FROM THE ACADEMY
Standards of Practice

Academy of Nutrition and Dietetics: Revised 2021 Standards of Practice and Standards of Professional Performance for Registered Dietitian Nutritionists (Competent, Proficient, and Expert) in Sports and Human Performance Nutrition

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ABSTRACT

Nutrition in sports and human performance incorporates knowledge of the intersection of human physiology and nutrition. Registered dietitian nutritionist (RDN) practitioners in sports and human performance focus on nutrition care that is specific to the individual and their sport/occupational requirements. The Dietitians in Sports, Cardiovascular and Wellness Dietetic Practice Group, along with the Academy of Nutrition and Dietetics Quality Management Committee, have updated the Standards of Practice (SOP) and Standards of Professional Performance (SOPP) for RDNs working in sports and human performance. The SOP and SOPP for RDNs in Sports and Human Performance Nutrition provide indicators that describe three levels of practice: competent, proficient, and expert. The SOP uses the Nutrition Care Process and clinical workflow elements for delivering care to athletic/professional populations. The SOPP describes the following six domains that focus on professional performance: Quality in Practice, Competence and Accountability, Provision of Services, Application of Research, Communication and Application of Knowledge, and Utilization and Management of Resources. Specific indicators outlined in the SOP and SOPP depict how these standards apply to practice. The SOP and SOPP are complementary resources for RDNs and are intended to be used as a self-evaluation tool for assuring competent practice in sports and human performance and for determining potential education and training needs for advancement to a higher practice level in a variety of settings.

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Editor's note: Figures 1 and 2 that accompany this article are available online at www.jandonling.org.

THE SPORTS, CARDIOVASCULAR, and Wellness Nutrition (SCAN) Dietetic Practice Group (DPG) of the Academy of Nutrition and Dietetics (Academy), under the guidance of the Academy Quality Management Committee, has revised the Standards of Practice (SOP) and Standards of Professional Performance (SOPP) for Registered Dietitian Nutritionists (RDNs) in Sports Nutrition and Dietetics, which was originally published in 2014.[1] The revised document, Academy of Nutrition and Dietetics: Revised 2021 Standards of Practice and Standards of Professional Performance for Registered Dietitian Nutritionists (Competent, Proficient, and Expert) in Sports and Human Performance (SHP) Nutrition, reflects advances in sports and human performance nutrition practice during the past 7 years and replaces the 2014 Standards. This document builds on the Academy of Nutrition and Dietetics: Revised 2017 SOP in Nutrition Care and SOPP for RDNs.[2] The Academy of Nutrition and Dietetics/Commission on Dietetic Registration's (CDR) Code of Ethics for the Nutrition and Dietetics Profession,[3] along with the Academy of Nutrition and Dietetics: Revised 2017 SOP in Nutrition Care and SOPP for RDNs[2] and Revised 2017 Scope of Practice for the RDN,[4] guide the practice and performance of RDNs in all settings.

Scope of practice in nutrition and dietetics is composed of statutory and individual components, includes the code(s) of ethics (eg, Academy/CDR, other national organizations, or employers' code of ethics), and encompasses the range of roles, activities, practice guidelines, and regulations within which RDNs perform. For credentialed practitioners, scope of practice is typically established within the practice act and interpreted and

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Approved March 2021 by the Quality Management Committee of the Academy of Nutrition and Dietetics (Academy) and the Executive Committee of the Sports, Cardiovascular, and Wellness Nutrition Dietetic Practice Group of the Academy. ***Scheduled review date: April 2027.*** *Questions regarding the Standards of Practice and Standards of Professional Performance for Registered Dietitian Nutritionists in Sports and Human Performance Nutrition may be addressed to Academy Quality Management Staff: Dana Buelsing, MS, manager, Quality Standards Operations; and Karen Hui, RDN, LDN, scope/standards of practice specialist, Quality Management at quality@eatright.org.*

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controlled by the agency or board that regulates the practice of the profession in a given state.[4] An RDN's statutory scope of practice can delineate the services an RDN is authorized to perform in a state where a practice act or certification exists. For more information, see www.eatrightpro.org/advocacy/licensure/licensure-map.

The RDN's individual scope of practice is determined by education, training, credentialing, experience, and demonstrating and documenting competence to practice. Individual scope of practice in nutrition and dietetics has flexible boundaries to capture the breadth of the individual's professional practice. Professional advancement beyond the core education and supervised practice to qualify for the RDN credential provides RDNs practice opportunities, such as expanded roles within an organization based on training and certifications, if required; or additional credentials (eg, Certified Specialist in Sport Dietetics [CSSD]). The Scope of Practice Decision Algorithm (www.eatrightpro.org/scope) guides an RDN through a series of questions to determine whether a particular activity is within their scope of practice. The algorithm is designed to assist an RDN to critically evaluate their personal knowledge, skill, experience, judgment, and demonstrated competence using criteria resources.[5]

All registered dietitians are nutritionists—but not all nutritionists are registered dietitians. The Academy's Board of Directors and Commission on Dietetic Registration have determined that those who hold the credential Registered Dietitian (RD) may optionally use "Registered Dietitian Nutritionist" (RDN). The two credentials have identical meanings. In this document, the authors have chosen to use the term RDN *to refer to both registered dietitians and registered dietitian nutritionists.*

The Centers for Medicare and Medicaid Services, Department of Health and Human Services, Hospital[6] and Critical Access Hospital[7] Conditions of Participation now allow a hospital and its medical staff the option of including RDNs or other clinically qualified nutrition professionals within the category of "non-physician practitioners" eligible for ordering privileges for therapeutic diets and nutrition-related services if consistent with state law and health care regulations. RDNs in hospital settings interested in obtaining ordering privileges must review state laws (eg, licensure, certification, and title protection), if applicable, and health care regulations to determine whether any barriers or state-specific processes must be addressed. For more information, review the Academy's practice tips that outline the regulations and implementation steps for obtaining ordering privileges (www.eatrightpro.org/dietorders/). For assistance, refer questions to the Academy's State Affiliate organization.

Medical staff oversight of RDNs occurs in one of two ways. A hospital has the regulatory flexibility to appoint RDNs to the medical staff and grant RDNs specific nutrition ordering privileges, or can authorize the ordering privileges without appointment to the medical staff. To comply with regulatory requirements, an RDN's eligibility to be considered for ordering privileges must be through the hospital's medical staff rules, regulations, and bylaws, or other facility-specific process.[8] The actual privileges granted will be based on the RDN's knowledge, skills, experience, and specialist certification, if required, and demonstrated and documented competence.

Federal, state, and organization regulations, policies, and legally binding documents (eg, contracts) dictate the level of autonomous practice of an RDN in SHP. In the SHP setting, RDNs are empowered to initiate consults, recommend plans for food, hydration, and supplements, and to order laboratory tests within their individual scope of practice and facility privileges or protocols. However, RDN autonomy may be defined by designated medical providers responsible for overarching organization medical oversight.[9-13] SHP RDNs identify limitations to autonomous practice and work closely with designated medical leads and interprofessional team members to clearly define practice boundaries.

ACADEMY QUALITY AND PRACTICE RESOURCES

The Academy's Revised 2017 SOP in Nutrition Care and SOPP for RDNs[2] reflect the minimum competent level of nutrition and dietetics practice and professional performance. The core standards serve as blueprints for the development of focus area SOP and SOPP for RDNs in competent, proficient, and expert levels of practice. The SOP in Nutrition Care is composed of four standards consistent with the Nutrition Care Process and clinical workflow elements as applied to the care of patients, clients, or populations in all settings.[14] The SOPP consist of standards representing six domains of professional performance: Quality in Practice, Competence and Accountability, Provision of Services, Application of Research, Communication and Application of Knowledge, and Utilization and Management of Resources. The SOP and SOPP for RDNs are designed to promote the provision of safe, effective, efficient, equitable, and quality food and nutrition care and services; facilitate evidence-based practice; and serve as a professional evaluation resource.

These focus area standards for RDNs in SHP Nutrition provide a guide for self-evaluation and expanding practice, a means of identifying areas for professional development, and a tool for demonstrating competence in delivering SHP services. They are used by RDNs to assess their current level of practice and to determine the education and training required to maintain currency in their focus area and advancement to a higher level of practice. In addition, the standards can be used to assist RDNs in general clinical practice with maintaining minimum competence in the focus area and by RDNs transitioning their knowledge and skills to a new focus area of practice. Like the Academy's core SOP in Nutrition Care and SOPP for RDNs,[2] the indicators (ie, measurable action statements that illustrate how each standard can be applied in practice) (Figures 1 and 2, available at www.jandonline.org) for the SOP and SOPP for RDNs in SHP Nutrition were revised with input and consensus of content experts representing diverse practice and geographic perspectives. The SOP and SOPP for RDNs in SHP Nutrition were reviewed and approved by the Executive Committee of the SCAN DPG and the Academy Quality Management Committee.

THREE LEVELS OF PRACTICE

The Dreyfus model[16] identifies levels of proficiency (novice, advanced beginner, competent, proficient, and expert) (Figure 3) during the acquisition and development of knowledge and skills. The first two levels are components of the required didactic education (novice) and supervised practice experience (advanced beginner) that precede credentialing for nutrition and dietetics practitioners. On successfully attaining the RDN credential, a practitioner enters professional practice at the competent level and manages their professional development to achieve individual professional goals. This model is helpful in understanding the levels of practice described in the SOP and SOPP for RDNs in SHP Nutrition. In Academy focus areas, the three levels of practice are represented as competent, proficient, and expert; each level in SHP nutrition is associated with its own set of characteristics for the target population and the RDN (Figure 4). With the evolution of the field of SHP nutrition, RDNs may need to pursue additional knowledge (Figure 5) for application to practice or seek out a more experienced SHP RDN for assistance when an activity or complex case is beyond their experience level.

Sports and Human Performance practice involves working with all tiers of athletes/professionals. The competent-level RDN in SHP may enter the focus area through working with individuals engaging in sports for leisure and health benefits. An RDN in SHP working with competitive athletes[17] and individuals who are compensated for meeting minimum occupational standards is considered to be at the proficient level of practice (Figure 4). In addition to the practitioner progressing through the technical aspects of each level of competency, the SHP RDN should also be progressing through the ethical aspects of the profession. At each level of practice, the RDN must reflect on how personal biases may influence the interpretation of evidence[18] as well as cultural influences affecting their decision-making process.[3] This ongoing self-reflection is critical to ensuring fairness and objectivity that supports safe, equitable, timely, and appropriate treatment of individuals and organizations.[3] As the SHP RDN's level of practice advances, these principles must be applied to common SHP nutrition interactions such as contact with industry,[19] performance evaluations, candidate evaluations (eg, for employment, professional association memberships, awards, or scholarships), and allocation of resources.[3] Even the perception of differential treatment in these areas has the potential to compromise the best interest of organizations and individuals, making continual introspection a professional imperative. Regardless of level of practice, SHP RDNs act in a caring and respectful manner, mindful of individual differences and cultural and ethnic diversity[3] in all interactions with individuals and organizations.

With safe, efficient, effective, and equitable evidence-based practice[20] as guiding factors when working with athletes/professionals/organizations, the RDN identifies the level of evidence, clearly states research and source limitations, provides efficacy and safety information from reputable sources, and describes the risk and limitations of the intervention(s), when applicable. Sources of evidence include research, national and state guidelines, accreditation agencies, sport/occupational organization policies, consensus statements, systematic analysis of clinical experience, quality improvement data, specialized skills and knowledge of experts from SHP-related organizations (see Figure 5).

The Academy offers the Evidence Analysis Library (www.andeal.org/) as a resource, which provides a synthesis of systematic reviews on a variety of nutrition and dietetics topics, such as athletic performance, and physical activity.

The RDN is responsible for searching literature and assessing the level of evidence to select the best available evidence to inform recommendations. RDNs must evaluate and understand the best available evidence to converse authoritatively with the interprofessional team and adequately involve the athlete/professional or population in shared decision-making.

Competent Practitioner

In nutrition and dietetics, a competent practitioner is an RDN who is either just starting practice after having obtained RDN registration by CDR or an experienced RDN recently transitioning their practice to a new focus area of nutrition and dietetics. A focus area of nutrition and dietetics practice is a defined area of practice that requires focused knowledge, skills, and experience that apply to all levels of practice.[20] A competent practitioner who has achieved credentialing as an RDN and is starting in professional employment consistently provides safe and reliable services by employing appropriate knowledge, skills, behavior, and values in accordance with accepted standards of the profession; acquires additional on-the-job skills; and engages in tailored continuing education to further enhance knowledge, skills, and judgment obtained in formal education.[20]

A suggested starting point for an RDN interested in practicing in SHP nutrition is to gain exposure to target sport or occupation. Every sport or occupation is a miniature society in that it has its own culture, manifested through specific lingo, social norms, clothing, advancement pathways, role models, sponsorships, media coverage, and food or diet affinities and phobias. Approaches to assimilating to these nuances include, but are not limited to, reading books and articles, following social media, and attending events specific to target sport/occupation. Physical and psychological demands vary greatly across and within sports/occupation, making it critical that RDNs understand the basics of exercise metabolism and training theory to provide effective nutrition interventions. Working toward earning the CSSD certification, achieving the International Olympics Committee (IOC) Diploma in Sports Nutrition, completing college courses in sport nutrition and exercise physiology/kinesiology, and obtaining a general fitness certification are methods to achieving this requisite knowledge.

Proficient Practitioner

A proficient practitioner is an RDN who is generally 3 or more years beyond credentialing and entry into the profession and consistently provides safe and reliable service; has obtained operational job performance skills; and is successful in the RDN's chosen focus area of practice. The proficient practitioner demonstrates additional

Standards of Practice are authoritative statements that describe practice demonstrated through nutrition assessment, nutrition diagnosis (problem identification), nutrition intervention (planning, implementation), and outcomes monitoring and evaluation (four separate standards) and the responsibilities for which registered dietitian nutritionists (RDNs) are accountable. The Standards of Practice (SOP) for RDNs in Sports and Human Performance Nutrition presuppose that the RDN uses critical thinking skills; analytical abilities; theories; best-available research findings; current accepted nutrition, dietetics, and medical knowledge; and the systematic holistic approach of the nutrition care process as they relate to the application of the standards. Standards of Professional Performance (SOPP) for RDNs in Sports and Human Performance Nutrition are authoritative statements that describe behavior in the professional role, including activities related to Quality in Practice; Competence and Accountability; Provision of Services; Application of Research; Communication and Application of Knowledge; and Utilization and Management of Resources (six separate standards).

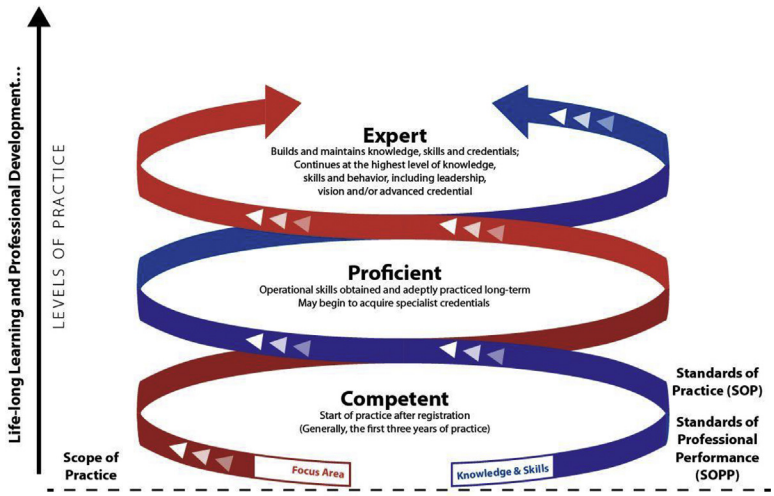
SOP and SOPP are complementary standards and serve as evaluation resources. All indicators may not be applicable to all RDNs' practice or to all practice settings and situations. RDNs operate within the directives of applicable federal and state laws and regulations as well as policies and procedures established by the organization in which they are employed and in which their clients are governed. To determine whether an activity is within the scope of practice of the RDN, the practitioner compares their knowledge, skill, experience, judgment, and demonstrated competence with the criteria necessary to perform the activity safely, ethically, legally, and appropriately. The Academy's Scope of Practice Decision Algorithm is specifically designed to assist practitioners with this process.

The term **athlete/professional** *is used in the SOP as a universal term, because these Standards relate to direct provision of nutrition care and services. Athlete/professional could also mean client or patient, resident, participant, consumer, or any individual or group who receives sports and human performance nutrition care and services.* **Customer** *is used in the SOPP as a universal term. Customer could also mean athlete/professional, client or patient, client, patient, or customer, participant, consumer, or any individual, group, or organization to which the RDN provides services. These services are provided to individuals of all ages. The SOP and SOPP are intended for all settings of sports and human performance nutrition, including clinical. In addition, it is recognized that the family, caregiver(s), and advocates of athletes/professionals of all ages, including individuals with special health care needs, play critical roles in overall health and are important members of the team throughout the assessment and intervention process. The term* **appropriate** *is used in the standards to mean: Selecting from a range of best practice or evidence-based possibilities, one or more of which would give an acceptable result in the circumstances.*

Each standard is equal in relevance and importance and includes a definition, a rationale statement, indicators, and examples of desired outcomes. A standard is a collection of specific outcome-focused statements against which a practitioner's performance can be assessed. The rationale statement describes the intent of the standard and defines its purpose and importance in greater detail. Indicators are measurable action statements that illustrate how each specific standard can be applied in practice. Indicators serve to identify the level of performance of competent practitioners and to encourage and recognize professional growth.

Standard definitions, rationale statements, core indicators, and examples of outcomes found in the Academy of Nutrition and Dietetics: Revised 2017 SOP in Nutrition Care and SOPP for RDNs have been adapted to reflect three levels of practice (competent, proficient, and expert) for RDNs in Sports and Human Performance Nutrition (see image below). In addition, the core indicators have been expanded to reflect the unique competence expectations for the RDN providing sports and human performance nutrition services.

Standards described as proficient level of practice in this document are not equivalent to the CDR certification, Board Certified as a Specialist in Sports Dietetics (CSSD). Rather, the CSSD designation recognizes the skill level of an RDN who has developed and demonstrated through successful completion of the certification examination, sports and human performance nutrition knowledge and application beyond the competent practitioner and demonstrates, at a minimum, proficient-level skills. An RDN with a CSSD designation is an example of an RDN who has demonstrated additional knowledge, skills, and experience in sports and human performance nutrition by the attainment of a specialist credential.



Adapted from the *Dietetics Career Development Guide*. For more information, please visit www.eatrightPRO.org/futurepractice

Figure 3. Standards of Practice and Standards of Professional Performance for Registered Dietitian Nutritionists (RDNs) (Competent, Proficient, and Expert) in Sports and Human Performance Nutrition.

Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance (SHP) Nutrition Practice Level Delineation		
Client/Work Focus		
Competent RDN in SHP	**Proficient RDN in SHP**	**Expert RDN in SHP**
Works with athletes/professionals who: • Participate in sport for leisure and are not compensated (recreational athletes) • Participate in training to enhance health and performance levels related to work tasks	Works with athletes/professionals who: • Must meet minimum qualifications, typically results or ranking, to compete (competitive athletes) • Must meet minimum performance standards to receive occupational compensation or stipend	In addition to working with athletes/professionals as described in competent and proficient, also works with organizations who: • Provide education and services to athletes/professionals • Conduct research • Develop products • Develop policies and protocols • Monitor program quality and compliance
Skill Development		
Competent RDN in SHP	**Proficient RDN in SHP**	**Expert RDN in SHP**
Focuses on expanding knowledge of: basic exercise physiology; training periodization and adaptation principles; metabolism relative to exercise intensity, duration, and modality; and sport- or occupation-specific organizational culture	Focuses on developing and expanding skills in: program development that support a comprehensive SHP nutrition concept (Figure 6), leadership, and mentoring	Focuses on integration of SHP nutrition into interprofessional teams and organizations; mentors RDNs in SHP; educates, and consults with other SHP professionals
Knowledge, Ability, and Experience		
Competent RDN in SHP	**Proficient RDN in SHP**	**Expert RDN in SHP**
• Knowledge of exercise metabolism and basics of SHP culture • Able to apply basic principles of SHP nutrition to athletes/professionals • 1-3 years' experience with individuals participating in recreational sports	Competent plus: • Knowledge of levels of athlete/professional and sport or occupation subtleties and culture and impact of athlete's/professional's performance on team or organization goals • Ability to forecast interconnecting factors and modify recommendations accordingly • 3—5 years' experience working with athletes/professionals/organizations and interprofessional performance enhancement teams	Proficient plus: • Knowledge of and ability to apply leadership principles to directing interprofessional teams, negotiating, conducting research, or mentoring RDNs in SHP • 5 or more years' experience working with athletes/professionals/organizations and interprofessional performance enhancement teams

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Figure 4. Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance (SHP) Nutrition Practice Level Delineation.

Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance (SHP) Nutrition Practice Level Delineation		
Career Advancement Opportunities		
Competent RDN in SHP	**Proficient RDN in SHP**	**Expert RDN in SHP**
• Pursues documented hours applicable toward SHP-related certification or credential such as o Certified Specialist in Sport Dietetics (CSSD) ○ International Olympic committee (IOC) Diploma in Sport Nutrition • Completes or currently enrolled in undergraduate or graduate-level courses in sports nutrition, exercise physiology/kinesiology • Completes SHP-applicable continuing education (CEU) courses, webinars, conferences • Obtains a general fitness credential (eg, Certified Personal Trainer, Group Fitness Instructor)	• Obtains SHP-related certification or credential such as: ○ CSSD ○ IOC Diploma in Sport Nutrition • Completes or currently enrolled in undergraduate or graduate-level coursework in sports nutrition, exercise physiology, or kinesiology • Completes SHP-applicable CEU courses, webinars, conferences • Obtains target population—specific physical training credential (eg, Triathlon or Weightlifting Level 1 Coach, Tactical Strength and Conditioning Facilitator) • Participates in SHP-specific professional organizations	• Maintains SHP-related certification or credential such as the CSSD • Completes graduate degree and/or completed or currently enrolled in graduate-level coursework in sports nutrition, exercise physiology, or kinesiology • Completes SHP-applicable and/or business, management, and leadership CEU courses, webinars, conferences • Obtains advanced SHP-relevant certification (eg, Certified Strength and Conditioning Specialist, International Society for the Advancement of Kinanthropometry) • Assumes leadership positions in occupational or SHP-specific professional organizations
Although SHP practice involves working with all tiers of athletes/professionals, working with competitive athletes and individuals who are compensated for meeting minimum occupational standards is considered to be at the proficient level of practice for RDNs working in SHP.		

Figure 4. *(continued)* Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance (SHP) Nutrition Practice Level Delineation.

knowledge, skills, judgment, and experience in a focus area of nutrition and dietetics practice.[20]

The proficient-level SHP practitioner may be working toward earning a SHP-related certification that is most applicable to their target athlete/population (eg, sport-specific level 1 coach or tactical strength and conditioning facilitator) to increase knowledge base and incorporate SHP concepts into practice. The RDN has an understanding of sport or occupation foundation and nuances and how to leverage those nuances for effective nutrition interventions (eg, need for a different nutrition strategy for a track vs a road cyclist or for a wildland vs a structural firefighter). The proficient-level RDN develops nutrition programs that support a comprehensive SHP nutrition approach (Figure 6) that takes into account the athlete's/professional's performance impact on the team's or organization's success and demonstrates understanding of influencing factors on nutrition intervention (eg, leadership and sponsorship influence and collective bargaining). Additionally, this level of practitioner balances short-term task-specific physical performance goals with long-term health and sport or occupation longevity. The RDN may complete undergraduate or graduate level coursework in sports nutrition or exercise physiology or kinesiology and obtain the CSSD or IOC Diploma in Sports Nutrition.

Expert Practitioner

An expert practitioner is an RDN who is recognized within the profession and has mastered the highest degree of skill in, and knowledge of, nutrition and dietetics. Expert-level achievement is acquired through ongoing critical evaluation of practice and feedback from others. The individual at this level strives for additional knowledge, experience, and training. An expert has the ability to quickly identify "what" is happening and "how" to approach the situation. Experts easily use nutrition and dietetics skills to become successful through demonstrating quality practice and leadership, and to consider new opportunities that build on nutrition and dietetics.[20] An expert practitioner may have an expanded or specialist role or both. Generally, the practice is more complex, and the practitioner has a high degree of professional autonomy and responsibility.

The expert-level SHP RDN applies leadership principles in directing interprofessional teams to ensure that nutrition education and services are

delivered in the comprehensive SHP nutrition approach (Figure 6). The expert-level RDN engages in negotiations concerning financial and human resources to assure adequate resources are available to deliver the SHP services. Additionally, the practitioner at the expert level of SHP nutrition conducts and applies research to advance the body of knowledge and the development of best practices. As a recognized leader in the field, the expert RDN mentors students, other RDNs, and professionals, as well as advocates for the inclusion of nutrition concepts and professionals in SHP. To retain an expert level of practice, the RDN maintains SHP-related certification(s) or credential(s), such as:

- the CSSD or Certified Eating Disorders Registered Dietitian (CEDRD);
- completed graduate-level coursework or continuing education opportunities in sport nutrition, exercise physiology/kinesiology, and/or eating disorders or disordered eating; and
- pursuit of advanced certifications such as the Certified Strength or Conditioning Specialist (CSCS) or the International Society for the Advancement of Kinanthropometry (ISAK).

Leadership and management skills are enhanced through completing business and leadership courses, webinars, and conferences.

These Standards, along with the Academy/CDR Code of Ethics,[3] answer the questions: Why is an RDN uniquely qualified to provide SHP nutrition services? What knowledge, skills, and competencies does an RDN need to demonstrate for the provision of safe, efficient, effective, equitable, and quality sports and human performance nutrition care and service at the competent, proficient, and expert levels?

OVERVIEW

Refer to alphabetical Glossary for definitions

Sports and Human Performance (SHP) Nutrition[21] is the practical application of nutrition science, food service, and culinary arts to sustained optimal execution of specific sport/occupation tasks (human performance[22]). Over the last three decades, the field of SHP nutrition, along with public interest, has evolved from a focus on nutrition for physical activity[23] in the support of health[24] to a focus on nutrition for training[25] in support of sports, human performance, and career longevity.[26-32] This general shift in focus from heath as an endpoint, to a focus on health as just one component of sports and human performance, has changed the way RDNs provide care and services in support of SHP nutrition. This evolved focus allows for a nuanced approach to culturally diverse nutrition recommendations that balance short-term performance optimization with long-term health and career longevity.[33]

SHP RDNs are uniquely qualified to provide services within a holistic SHP nutrition framework for all levels of athletes/professionals. This comprehensive approach includes proactive, active, and reactive components (Figure 6).[21] Proactive education and services establish foundational health through chronic disease prevention and immune system enhancement. Physical activity and appropriate nutrition facilitate foundational health by contributing to chronic disease and cancer prevention, cardiovascular and brain health, and appropriate body weight maintenance.[34,35] Emerging fields in nutrition approaches for foundational health include injury and illness prevention,[36] sleep enhancement,[37,38] and traumatic brain injury protection.[39,40]

Active component nutrition education and services support task- and environment-specific human performance through event fueling, post-event recovery, body composition management, cognitive performance enhancement, and arduous environment preparedness.[21,25] By using periodized nutrition[41] strategies to be integrated and synchronized with training cycles and competition or operational readiness or performance schedules, nutrition enhances training adaptations and recovery.[25,41] Short-term nutrition strategies to optimize SHP sometimes contradict long-term strategies for foundational health.[25,41] This balance between nutrition strategies for short-term physical and mental performance with strategies for long-term health and sport or occupation longevity require advanced knowledge by a SHP RDN.

Reactive component care and services address medical nutrition therapy and counseling to support illness and injury recovery[42] and long-term conditions. Medical nutrition therapy for acute and chronic diseases and conditions serve to expeditiously return athletes/professionals to sustainable activity/duty,[40] whereas nutrition in support of long-term conditions[43] (eg, impaired ability to thermoregulate in athletes/professionals with spinal cord injuries[44]) serves to optimize the performance of para athletes and persons with a physical impairment.[45]

By combining this holistic approach to SHP nutrition with the history, diversity, and current status of individuals and teams, RDNs develop and deliver care and services within the context of the athlete's/professional's training cycle,[24] competition or operational readiness or performance schedule,[24] organization culture,[46] and available resources.

The RDN Practicing in Sports and Human Performance Nutrition

Within the context of evidence-based practice,[47] the SHP RDN provides consultation to a range of athletes/professionals, including those with and without physical impairments who participate in recreational or competitive sport,[48,49] work in tactical occupations,[50,51] or perform in artistic fields.[52-55] The SHP RDNs work in settings such as colleges and universities[30]; athletic performance and training centers[56-58]; sports medicine facilities[30]; National Sport Governing Body (NGB); Olympic and professional sports organizations[59,60]; law enforcement and first responder departments[61,62]; government agencies[30]; military training and operational organizations[63,64]; and dance and performance companies. Potential future settings may include but are not limited to stunt production venues and e-sports (eg, competitive video gaming).

SHP RDNs view athletes/professionals holistically and consider multiple factors that affect nutritional intake and requirements when developing nutrition strategies. Individual athlete/professional assessment factors include food choice determinants (eg, physiological, biological, lifestyle, beliefs, knowledge, psychological, social, and economic),[65] health and fitness

Resource	Address	Description
General Health, Sport, and Human Performance		
Academy of Nutrition and Dietetics (Academy) Sports and Human Performance Nutrition Dietetic Practice Group (DPG) and Cardiovascular Health and Well-being DPG	https://www.eatrightpro.org/membership/academy-groups/dietetic-practice-groups	The Sports, Cardiovascular and Wellnesss Nutrition (SCAN) DPG has expanded into two new DPGs beginning membership year 2021-2022; the Sports and Human Performance Nutrition (SHPN) DPG and the Cardiovascular Health and Well-being (CV-Well) DPG. Both DPGs will provide archived SCAN materials (eg, fact sheets and the Pulse newsletter) as well as resources relevant to their membership.
Academy Fitness webpage	https://www.eatright.org/fitness	This Academy webpage gives consumer information on: • fitness • sports, • performance, • training, and • recovery.
American College of Sports Medicine (ACSM)	www.acsm.org	This organization provides: • science-backed information and practical application in the fields of exercise science and sports medicine; • certifications, position papers, journals, and policy advocacy; • forums for focused discussion, activity, and debate; and Special Interest Groups for nutrition, combat sports, endurance athletes, team sports, health and performance, military, and performing arts.
Food & Drug Administration (FDA)	https://www.fda.gov/food/dietary-supplements	Contains consumer and industry information on dietary supplement regulation, including steps to report dietary supplement adverse events.
Food & Drug Administration (FDA) Tainted Products Marketed as Dietary Supplements database	https://www.accessdata.fda.gov/scripts/sda/sdNavigation.cfm?sd=tainted_supplements_cder&displayAll=false&page=6	The FDA's searchable database includes previously identified products with hidden ingredients marketed to consumers as dietary supplements; not all inclusive.
National Athletic Trainers' Association (NATA)	https://www.nata.org	The organization provides: • evidence-based information on heatlh care topics relative to sports and human performane. • educational workshops and continuing education opportunities. • position statements and scholarly publications. • cultural sensitivity resources. • career opportunities.

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Figure 5. Resources for Registered Dietitian Nutritionists in Sports and Human Performance Nutrition (not all inclusive).

Resource	Address	Description
National Strength and Conditioning Association (NSCA)	https://www.nsca.com/	This association offers: • research-based information and its practical application of strength and conditioning through certifications, research journals, career development services, and continuing education opportunities; and a forum for engagement with others in Special Interest Groups for nutrition, metabolism, body composition, specific sports, performing arts, sports medicine, and tactical occupations.
Position of the Academy of Nutrition and Dietetics, Dietitians of Canada, and the American College of Sports Medicine: Nutrition and Athletic Performance	https://www.eatrightpro.org/practice/position-and-practice-papers/position-papers/new-position-papers-and-joint-papers-with-other-organizations	This position paper gives consensus guidelines on: • the type, amount, and timing of intake of food, fluids, and dietary supplements to support athletic performance; and • sport performance across training and competitive sport.
Professionals in Nutrition for Exercise and Sport	https://pinesnutrition.org/	This organization offers members: • SHP articles for RDNs; and • information on careers and education in sports nutrition
Collegiate, Olympic, and Professional Sport Performance		
Australian Institute of Sport	https://ais.gov.au/nutrition	This website provides: • links to full-text evidence-based articles on nutrition, fluid replacement, dietary supplements, relative energy deficiency in sport (RED-S), nutrition support of immunity, and body image; • evidence-based dietary supplement classification system that ranks sports foods and supplement ingredients to determine safety and effectiveness; • fact sheets on multiple sport nutrition topics; and • performance-based recipes
Collegiate and Professional Sports Dietitians Association	https://www.sportsrd.org/	This association has information for registered dietitian nutritionists in the United States who work with athletes in colleges and universities, professional sports, Olympic training centers, the U.S. Military, and in law enforcement.
National Collegiate Athletic Association (NCAA)	http://www.ncaa.org/	This website includes: • information about policy regarding the provision of on-campus meals and snacks to collegiate student-athletes; and • information on sleep, nutrition, and dietary supplements for student-athletes, coaches, and administrators

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Figure 5. *(continued)* Resources for Registered Dietitian Nutritionists in Sports and Human Performance Nutrition (not all inclusive).

Resource	Address	Description
U.S. Anti-Doping Agency (USADA) Substances webpage	https://www.usada.org/athletes/substances/	This USADA webpage provides a search function for ingredients identified by the World Anti-Doping Agency as prohibited.
U.S. Olympic & Paralympic Committee	https://www.teamusa.org/nutrition	This website provides: • print ready education material on easy, moderate, and hard training day plate guides for use with athletes; • fact sheets on multiple sports nutrition topics; and • performance-based recipes
World Anti-Doping Agency (WADA)	https://www.wada-ama.org/	The WADA provides an annually updated list of substances and methods that are prohibited to athletes in- and out-of-competition.
Tactical Performance		
Uniform Services University (USU) Human Performance Resources	https://www.hprc-online.org/nutrition/performance-nutrition	This webpage from the Consortium for Health and Military Performance (CHAMP) provides nutrition basics and specialty topics to enhance Service Member performance during workouts, training, missions, and recovery.
Uniform Services University (USU) Operation Supplement Safety webpage	https://www.opss.org/	This webpage from the Department of Defense (DoD) provides: • a DoD prohibited dietary supplement list; • safety screening tool for dietary supplements; • link to Food and Drug Administrtion (FDA) Safety Reporting Portal to report dietary supplement side effects; and • updated links to FDA alerts and announcements regarding dietary supplements, ingredients, and fraudulent products
Artistic Performance		
Athletes and the Arts	http://athletesandthearts.com/	This website introduces RDNs to the culture of performing artists as athletes.
Performing Arts Medicine Association	http://www.artsmed.org/	This website introduces RDNs to events focused on the health and performance of performing artists.

Figure 5. *(continued)* Resources for Registered Dietitian Nutritionists in Sports and Human Performance Nutrition (not all inclusive).

status,[66] medical history, environmental, performance targets, body composition, food preference,[67] and career transition.[68] Critical to the assessment, SHP RDNs assess associations between histories of training and performance outcomes, injury, and illness with histories of diet, body composition, and dietary supplement[20] use. Sport- or occupation-specific factors include physiological requirements,[22] regulations, culture,[46] training cycles, competition/operational readiness or performance schedules,[25] environment, and travel.[67] Sport or occupation team factors include food preferences of the team, catering arrangements (or lack of), resource limitations (ie, money, personnel, time), team dietary supplement use, sponsorships,[67] and culinary skill set. The impact of additional influencers on nutrition intake of both individuals and teams, including coaches, leadership, teammates, peers, and family, are assessed.

SHP RDNs combine this assessment with a thorough understanding of periodized nutrition[25,41] to develop effective nutrition strategies that address proactive, active, and reactive components throughout the athlete's/professional's competitive/operational

readiness or performance schedule.[29] Education, anthropometric and biochemical assessments, as well as nutrient intake plans, are synchronized with the primary goals of each training session and phase.[25,33]

RDNs working in SHP nutrition maintain the delicate balance between short-term performance optimization with long-term health and sport or occupation longevity. Examples of contradictory short-term strategies include:

- decreasing energy or macronutrient intake or availability to enhance training adaptations or cognitive performance enhancement,[30,41,69]
- modifying body fat composition to levels traditionally considered unhealthy to meet weight class or aesthetic requirements,[70-72]
- anticipating periods of food insufficiency (eg, military survival training or accelerated fuel stores depletion),[72-75]
- deliberate nonadherence to recommended Adequate Intake (AI) or Tolerable Upper Intake Levels (UL) for nutrients (or vitamins or minerals) to prepare for event-specific environmental limitations (eg, fiber intake below AI for operations with restricted bowel evacuation opportunities or sodium intake above UL for events occurring in excessive heat and humidity).

As a proactive and integral member of an organization's human performance program[21,75] and performance enhancement team,[76-78] RDNs provide education, services, and care that are integrated and synchronized with training cycles, competition or operational readiness or performance schedules, and the efforts of other interprofessional team members. Thorough knowledge of performance enhancement team members' scopes of practice, areas of expertise, and style of practice facilitates interprofessional team effectiveness and referrals that are seamless and timely.[79-81] SHP RDNs balance athlete/professional confidentiality mandated by the Health Insurance Portability and Accountability Act of 1996 with providing status updates to coaches, leadership, and decision makers regarding athlete's/professional's health and performance status. A comprehensive understanding of the athlete's/professional's role and contribution to their organization enables the RDN to evaluate return to activity or duty implications for both the athlete/professional and the organization.[82]

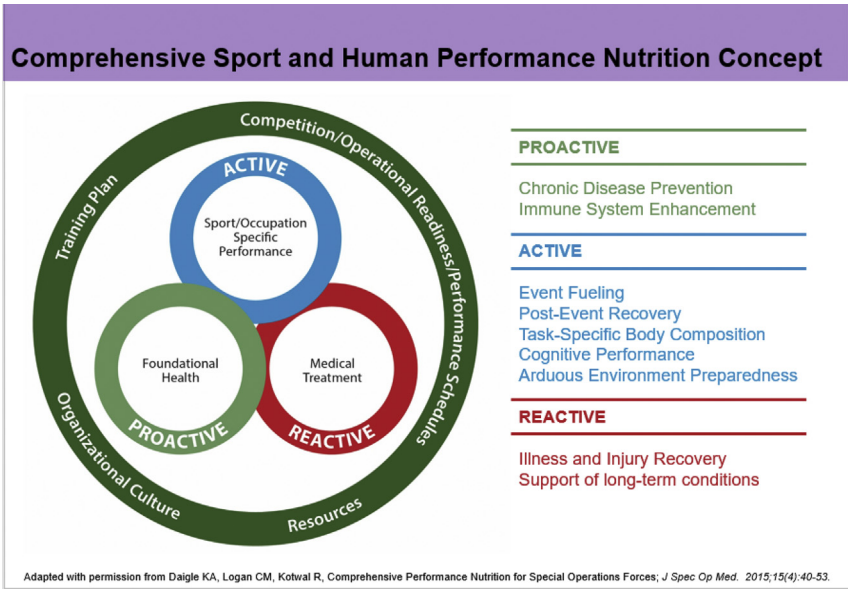

Figure 6. Comprehensive Sport and Human Performance Nutrition Concept.[21]

ACADEMY REVISED 2021 SOP AND SOPP FOR RDNs (COMPETENT, PROFICIENT, AND EXPERT) IN SPORTS AND HUMAN PERFORMANCE NUTRITION

An RDN can use the Academy Revised 2021 SOP and SOPP for RDNs (Competent, Proficient, and Expert) in Sports and Human Performance Nutrition (Figures 1 and 2, available at www.jandonline.org, and Figure 3) to:

- identify the competencies needed to provide SHP nutrition care and services;
- self-evaluate whether they have the appropriate knowledge, skills, cultural competencies, and judgment to provide safe, efficient, effective, equitable and quality SHP nutrition care and service for their level of practice;
- identify the areas in which additional knowledge, skills, and experience are needed to practice at the competent, proficient, or expert level of SHP nutrition practice;
- provide a foundation for public and professional accountability in SHP nutrition care and services;
- support efforts for strategic planning, performance improvement, and outcomes reporting, and assist management in the planning and communicating of SHP nutrition services and resources;
- enhance professional identity and skill in communicating the nature of SHP nutrition care and services;
- guide the development of diverse SHP nutrition-related education and continuing education programs, job descriptions, practice guidelines, protocols, clinical models, competence evaluation tools, and career pathways; and
- assist educators and preceptors in teaching students and interns the knowledge, skills, and culturally informed practice-based competencies needed to work in SHP nutrition, and the understanding of the full scope of this focus area of practice.

APPLICATION TO PRACTICE

All RDNs, even those with significant experience in other practice areas, must begin at the competent level

Role	Examples of use of SOP and SOPP documents by RDNs in different practice roles[a]
Collegiate sports team practitioner	An RDN new to a collegiate team environment is responsible for providing sport-specific nutrition education, individual and team meal planning, and ancillary services to support extensive training, travel, and play with which the RDN has limited knowledge and experience. The RDN uses the SOP and SOPP for RDNs in Sports and Human Performance (SHP) Nutrition as a guide to ensure their service is at the appropriate level of practice, and to identify areas that may require collaboration with other SHP professionals that have more experience with this specific population, environment, and culture.
Human performance program manager or collegiate athletic director	An RDN who oversees an interprofessional human performance team wants to ensure the RDN team members are being used appropriately. The director uses the SOP and SOPP for RDNs in SHP Nutrition to ensure duties are appropriately assigned, that individual RDNs are functioning at the appropriate level of practice required for the program's population, and to identify continuing training and experiences required to develop and maintain a high level of practice.
Consulting practitioner	An RDN in private practice who has been consulting with recreational athletes is contacted by a university to provide SHP nutrition services to its scholarship athletes via telehealth. The RDN uses the SOP and SOPP for RDNs in SHP Nutrition to determine additional competencies required when providing services to this level of competitive athletes and for determining any additional considerations for delivering services via telehealth. The RDN refers to all relevant state laws and regulations, the Academy of Nutrition and Dietetics telehealth resources, and organization policies regarding the practice of telehealth, including requirements if the athlete/professional lives in another state. The SOP and SOPP in SHP Nutrition is used to evaluate competence and identify areas for additional knowledge or when an RDN with more expertise in SHP nutrition needs to be consulted or to make a referral for assuring quality care.
Private practice practitioner	An RDN in private practice who provides a range of services for physically active individuals has decided to offer consulting to firefighter and police agencies. The RDN uses the SOP and SOPP for RDNs in SHP Nutrition to evaluate the knowledge, skills and competencies required to provide quality services to this tactical population (eg, interpersonal skills and assessment/intervention competency). The RDN sets goals to increase tactical SHP knowledge base and builds a network of mentors and colleagues to improve competency before providing services independently to this population.
Military practitioner	An RDN working in a military environment is directed to provide nutrition education and services in support of an operational unit's field training event with which they have limited knowledge and experience. The RDN uses the SOP and SOPP for RDNs in SHP Nutrition as a guide to ensure their organization intervention is at the appropriate level of practice and to identify areas that may require collaboration with other SHP or military professionals that have more experience with this specific population and event.
Clinical practitioner, outpatient care	An RDN working in a health care facility with an outpatient clinic that offers education and services to local recreational sports teams and athletes, has been asked to provide nutrition and education services. The RDN reviews available resources and the SOP and SOPP for RDNs in SHP Nutrition to evaluate the knowledge, skills and competencies required to provide quality services to individuals participating in recreational sports (eg, nutrient timing, recovery nutrition). The RDN sets goals to increase their SHP knowledge base and builds a network of mentors and colleagues to improve competence before providing services independently to this population.

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Figure 7. Role Examples of Standards of Practice (SOP) and Standards of Professional Performance (SOPP) for Registered Dietitian Nutritionists (RDNs) (Competent, Proficient, and Expert) in Sports and Human Performance (SHP) Nutrition.

Role	Examples of use of SOP and SOPP documents by RDNs in different practice roles[a]
Professional nutrition and dietetics writer	An RDN freelance writer is commissioned to write an article on sports nutrition for a nationally distributed fitness magazine. The RDN reviews available resources regarding SHP nutrition applicable to the publication's target audience and identifies knowledge deficits. The RDN refers to the SOP and SOPP for SHP Nutrition, and consults with proficient- or expert-level SHP nutrition RDNs, to identify knowledge needed, and resources available, to provide evidence-based information that would benefit the publication's audience.
Researcher	An RDN working in a research setting is awarded a grant to demonstrate the impact of SHP nutrition services on athletic and tactical human performance program outcomes. The RDN consults with proficient- and expert-level SHP nutrition RDNs in designing the research protocol. The RDN uses the SOP and SOPP as tools for identifying key resources and for identifying when to collaborate with an RDN more experienced in SHP nutrition research.
[a]For each role, the RDN updates their professional development plan to include applicable essential practice competencies for SHP nutrition care and services.	

Figure 7. *(continued)* Role Examples of Standards of Practice (SOP) and Standards of Professional Performance (SOPP) for Registered Dietitian Nutritionists (RDNs) (Competent, Proficient, and Expert) in Sports and Human Performance (SHP) Nutrition.

when practicing in a new setting or new focus area of practice. At the competent level, an RDN in SHP nutrition is learning the principles that underpin this focus area and is developing knowledge, skills, and judgment, and gaining experience for safe and effective SHP nutrition practice. This RDN, who may be new to the profession or may be an experienced RDN, has a breadth of knowledge in nutrition and dietetics and may have proficient or expert knowledge or practice in another focus area. However, the RDN new to the focus area of SHP nutrition must accept the challenge of becoming familiar with the body of knowledge, practice guidelines, and available resources to support and ensure quality SHP nutrition-related practice. At the competent level, the RDN works with:

- athletes/professionals through volunteer and paid experiences in SHP fields;
- noncompensated recreational athletes[83] through intramural clubs, high school programs, recreational facilities, rehabilitation programs, internships programs, and event organizations; and
- tactical and artistic professionals through Reserve Officer Training Corps programs, recruiting events, community programs, and internship opportunities.

At the proficient level, an RDN has developed a more in-depth understanding of SHP nutrition practice and is more skilled at adapting and applying evidence-based guidelines and best practices than at the competent level. At the proficient level, the RDN works with competitive athletes (ie, those who must meet minimum qualifications to compete) through positions with collegiate, professional, Olympic/Paralympic, and other national-level sport programs and training centers. The RDN works with tactical and artistic professionals through training units or academies, operational units' or departments' human performance programs, and performing arts organizations. The RDN at the proficient level may possess a specialist credential(s) such as the CSSD. By identifying the RDN as having specialized SHP nutrition knowledge, the CSSD credential has become a sought-after credential for professional SHP jobs.

At the expert level, the RDN thinks critically about SHP nutrition, demonstrates a more intuitive understanding of the practice area, displays a range of highly developed clinical and technical skills, and formulates judgments acquired through a combination of education, experience, and critical thinking. Essentially, practice at the expert level requires the application of nutrition, biochemistry, exercise physiology, and sport- or occupation-specific knowledge, with practitioners drawing not only on their practice experience, but also on the experience of other SHP RDNs in various disciplines and practice settings. Expert RDNs, with their extensive experience and ability to see the significance and meaning of SHP nutrition within a contextual whole, are fluid and flexible and have considerable autonomy in practice. The expert-level RDN applies leadership principles in directing interprofessional teams to ensure that nutrition education and services are delivered in the comprehensive SHP nutrition approach (Figure 6). Additionally, the practitioner at the expert level of SHP nutrition conducts and applies research to advance the body of knowledge and the development of best practices. As a recognized leader in the field, the expert RDN mentors students, other RDNs, and professionals, as well as advocates for nutrition content and professionals in SHP. To retain an expert level of practice, the RDN maintains SHP-related certification(s) or credential(s), such as the CSSD, may complete graduate-level coursework or continuing education opportunities in sport nutrition or exercise physiology or kinesiology, and may pursue advanced certifications such as CSCS or ISAK. Leadership and management skills are enhanced through completing business and leadership courses, webinars, and conferences.

Indicators for the SOP and SOPP for RDNs in SHP Nutrition are measurable action statements that illustrate how each standard can be applied in practice (Figures 1 SOP and 2 SOPP, available at www.jandonline.org). Within the SOP and SOPP for RDNs in SHP Nutrition, an "X" in the competent

column indicates that an RDN who is caring for athletes/professionals is expected to complete this activity or seek assistance to learn how to perform at the level of the standard. A competent RDN in SHP nutrition could be an RDN starting practice after registration or an experienced RDN who has recently assumed responsibility to provide SHP nutrition care for athletes/professionals as described in Figure 4.

An "X" in the proficient column indicates that an RDN who performs at this level has a deeper understanding of SHP nutrition and has the ability to modify or guide therapy to meet the needs of athletes/professionals, with or without medical conditions, at all levels and phases of sport or occupation.

An "X" in the expert column indicates that the RDN who performs at this level possesses a comprehensive understanding of SHP nutrition and a highly developed range of skills and judgments acquired through a combination of experience and education. The expert RDN builds and maintains the highest level of knowledge, skills, and behaviors, including leadership, vision, and credentials (Figure 4).

Standards and indicators presented in Figure 1 and Figure 2 (available at www.jandonline.org) in boldface type originate from the Academy's Revised 2017 SOP in Nutrition Care and SOPP for RDNs[2] and should apply to RDNs in all three levels. Additional indicators not in boldface type developed for this focus area are identified as applicable to all levels of practice. Where an "X" is placed in all three levels of practice, it is understood that all RDNs in SHP nutrition are accountable for practice within each of these indicators. However, the depth with which an RDN performs each activity will increase as the individual moves beyond the competent level. Several levels of practice are considered in this document; thus, taking a holistic view of the SOP and SOPP for RDNs in SHP Nutrition is warranted. It is the totality of individual practice that defines a practitioner's level of practice, and not any one indicator or standard.

RDNs should review the SOP and SOPP in SHP Nutrition at determined intervals to evaluate their individual focus area knowledge, skill, and competence. Consistent self-evaluation is important because it helps identify opportunities to improve and enhance practice and professional performance and set goals for professional development. This self-appraisal also enables SHP nutrition RDNs to better use these Standards as part of the *Professional Development Portfolio* recertification process,[84] which encourages CDR-credentialed nutrition and dietetics practitioners to incorporate self-reflection and learning needs assessment for development of a learning plan for improvement and commitment to lifelong learning. CDR's updated system implemented with the 5-year recertification cycle that began in 2015 incorporates the use of essential practice competencies for determining professional development needs.[85] In the new three-step process, the credentialed practitioner accesses the Competency Plan Builder[86] (step 1), which is a digital tool that assists practitioners in creating a continuing professional education Learning Plan. It helps identify focus areas during each 5-year recertification cycle for verified CDR credentialed nutrition and dietetics practitioners. The Activity Log (step 2) is used to log and document continuing professional education over the 5-year period. The Professional Development Evaluation (step 3) guides self-reflection and assessment of learning and how it is applied. The outcome is a completed evaluation of the effectiveness of the practitioner's learning plan and continuing professional education. The self-assessment information can then be used in developing the plan for the practitioner's next 5-year recertification cycle. For more information, see https://www.cdrnet.org/competencies-for-practitioners.

RDNs are encouraged to pursue additional knowledge, skills, and training, regardless of practice setting, to maintain currency and to expand their individual scope of practice within the limitations of the legal scope of practice, as defined by state law. RDNs are expected to practice only at the level at which they are competent, and this will vary depending on education, training, and experience.[87] RDNs should collaborate with other RDNs in SHP nutrition as learning opportunities and to promote consistency in cultural competencies, practice, performance, and continuous quality improvement. Figure 7 gives role examples of how RDNs in different roles, at different levels of practice, may use the SOP and SOPP in SHP Nutrition.

In some instances, components of the SOP and SOPP for RDNs in SHP Nutrition do not specifically differentiate between proficient-level and expert-level practice. In these areas, it remains the consensus of the content experts that the distinctions are subtle, captured in the knowledge, experience, and intuition demonstrated in the context of practice at the expert level, which combines dimensions of understanding, performance, and value as an integrated whole.[88] A wealth of knowledge is embedded in the experience, discernment, and practice of expert-level RDN practitioners. The experienced practitioner observes events, analyzes them to make new connections between events and ideas, and produces a synthesized whole. The knowledge and skills acquired through practice will continually expand and mature. The SOP and SOPP indicators are refined with each review of these Standards as expert-level RDNs systematically record and document their experiences, often through use of exemplars. Exemplary actions of SHP nutrition RDNs in practice settings and professional activities that enhance athlete/professional or organization care or services can be used to illustrate outstanding practice models.

FUTURE DIRECTIONS

The SOP and SOPP for RDNs in SHP Nutrition are innovative and dynamic documents. Future revisions will reflect changes and advances in practice, changes to dietetics education standards, regulatory changes, evidence-based guidelines,[47] and outcomes of practice audits. Continued clarity and differentiation of the three practice levels in support of safe, effective, and quality practice in SHP nutrition remains an expectation of each revision to serve tomorrow's practitioners and their athletes/professionals or organizations. Sports and Human Performance is a rapidly advancing field in which nutrition is poised to play an increasingly critical role. Through integration and synchronization with training cycles, competition, operational readiness, performance schedules, and efforts of other SHP disciplines and professionals, RDNs have the opportunity to profoundly impact the health, quality of life, success, and career longevity of athletes/professionals.

The evolution of the field of SHP Nutrition has necessitated a concomitant evolution of the SCAN DPG. Therefore, the SCAN DPG has expanded into two new DPGs, beginning membership year 2021–2022. The Sports and Human Performance Nutrition DPG will be a network of credentialed nutrition and dietetics practitioners working with athletes and individuals at all levels to optimize human performance and holistic health for a lifetime. The Sports and Human Performance Nutrition DPG will deliver content tailored for human performance in sports as well as for in tactical occupations and the performing arts. Cardiovascular Health and Well-being DPG will equip members to be the experts promoting the prevention and treatment of cardiovascular disease through nutrition, well-being, and physical activity. The Cardiovascular Health and Well-being DPG will provide in-depth content in cardiovascular nutrition and well-being so members can optimize their work in diverse health arenas such as clinical settings, corporate well-being, community nutrition, private practice, research, education, and government.[89]

SUMMARY

RDNs face complex situations every day. Addressing the unique needs of each situation and applying standards appropriately and equitably is essential to providing safe, timely, person-centered quality care and service. All RDNs are advised to conduct their practice based on the most recent edition of the Code of Ethics for the Nutrition and Dietetics Profession, the Scope of Practice for RDNs, and the SOP in Nutrition Care and SOPP for RDNs, along with applicable federal and state regulations and facility accreditation standards. The SOP and SOPP for RDNs in SHP Nutrition are complementary documents and are key resources for RDNs at all knowledge and performance levels. These standards can and should be used by RDNs in daily practice who provide care to individuals in SHP nutrition to consistently improve and appropriately demonstrate competence and value as providers of safe, efficient, effective, equitable, and quality nutrition and dietetics care and services. These standards also serve as a professional resource for self-evaluation and professional development for RDNs specializing in SHP nutrition practice. Just as a professional's self-evaluation and continuing education process is an ongoing cycle, these standards are also a work in progress and will be reviewed and updated every 7 years.

Current and future initiatives of the Academy, as well as advances in SHP nutrition care and services, will provide information to use in future updates and in further clarifying and documenting the specific roles and responsibilities of RDNs at each level of practice. As a quality initiative of the Academy and the SCAN Dietetic Practice Group, these standards are an application of continuous quality improvement and represent an important collaborative endeavor.

These standards have been formulated for use by individuals in self-evaluation, practice advancement, development of practice guidelines and specialist credentials, and as indicators of quality. These standards do not constitute medical or other professional advice, and should not be taken as such. The information presented in the standards is not a substitute for the exercise of professional judgment by the credentialed nutrition and dietetics practitioner. These standards are not intended for disciplinary actions, or determinations of negligence or misconduct. The use of the standards for any other purpose than that for which they were formulated must be undertaken within the sole authority and discretion of the user.

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GLOSSARY

Artistic fields are trades and occupations that involve a physical performance for audience entertainment, such as dancers, aerial artists, noncompetitive gymnastics, and stunt performers.[52-55]

A *competitive athlete* is an individual who "participates in an organized team or individual sport that requires regular competition against others as a central component, places a high premium on excellence and achievement, and requires some form of systematic (and usually intense) training."[17]

Female athlete triad refers to the interrelationships among energy availability, menstrual function, and bone mineral density, which may have clinical manifestations, including functional hypothalamic amenorrhea and osteoporosis.[81,90]

Health is a "state of complete physical, mental, and social well-being and not merely the absence of disease or infirmity."[24]

Human performance involves the "accomplishment of a given task measured against preset known standards."[22]

Human performance program is the systematic, integrated, and synchronized application of knowledge, skills, and emerging technologies by interprofessional performance enhancement teams to improve and sustain the health and task-specific performance of athletes/professionals and organizations.[21,75]

An *interprofessional research team* within athletic performance/sports settings may include any or all of the following: principal investigator, co-principal investigators, project consultants, laboratory technicians, and statisticians.

Periodization is a "logical method of planning training interventions in a sequential and integrative fashion to maximize training-induced physiological and performance outcomes."[91]

Periodized nutrition is the "planned delivery of specific nutritional interventions to enhance the adaptations targeted by individual exercise sessions or periodic training plans, or to obtain other effects that will enhance performance longer term."[41]

A *performance enhancement team* is an interprofessional group of professionals whose focus is on athlete/professional health and enhancement of human performance. Team members include human performance professionals and licensed medical providers, such as sport coaches, strength and conditioning coaches, physiologists, biomechanists, technologists, data scientists, physicians, chiropractors, physical therapists, registered dietitian nutritionists, chefs, psychologists, cognitive enhancement specialists, athletic trainers, acupuncturists, and massage therapists.[76-78]

Physical activity is "any bodily movement produced by skeletal muscles that requires energy expenditure."[23] Exercise and training are types of physical activity.

Professional, in the context of sports and human performance, is an individual who must perform employment/work tasks and may or may not be required to meet minimum performance standards to receive occupational compensation or a stipend (eg, firefighters, armed forces service members, dancers).

A *recreational athlete* is an individual who "is physically active but who does not train for competition at the same level of intensity and focus as a competitive athlete."[83]

Relative Energy Deficiency in Sport (RED-S) refers to impaired physiological function, including but not limited to metabolic rate, menstrual function, bone health, immunity, protein synthesis, cardiovascular health caused by relative energy deficiency.[70]

Sport is an "athletic activity requiring skill or physical prowess and often of a competitive nature."[49]

Sports and human performance nutrition is the practical application of nutrition science, food service, and culinary arts to the sustained optimal execution of specific sport/occupation tasks.[21]

Tactical occupations include military, fire and rescue, law enforcement, protective services, and other emergency personnel fields.[50,51]

Task-specific physical performance encompasses physical activity related to sport, military training and military operations, and training and performance of law enforcement officers, firefighters, professional emergency responders, and others whose occupations require physical labor or maintenance of specified levels of physical conditioning or body weight and body composition.

Training is the process of physically and mentally preparing for task-specific performance that typically includes strength and conditioning, skill acquisition, nutrition, psychological skills, and recovery.[25]

Standards of Practice for Registered Dietitian Nutritionists in Sports and Human Performance Nutrition
Standard 1: Nutrition Assessment
The registered dietitian nutritionist (RDN) in sports and human performance (SHP) nutrition uses accurate and relevant data and information to identify nutrition-related problems.
Rationale:
Nutrition screening is the preliminary step to identify individuals who require a nutrition assessment performed by an RDN. Nutrition assessment is a systematic process of obtaining and interpreting data to make decisions about the nature and cause of nutrition-related problems and provides the foundation for nutrition diagnosis. It is an ongoing, dynamic process that involves not only initial data collection, but also reassessment and analysis of athlete/professional or population needs. Nutrition assessment is conducted using validated tools based in evidence, the five domains of nutrition assessment, and comparative standards. Nutrition assessment may be performed via in-person, facility/practitioner assessment application, or Health Insurance Portability and Accountability Act (HIPAA) compliant video conferencing telehealth platform.

Indicators for Standard 1: Nutrition Assessment						
Bold font indicators are Academy Core RDN Standards of Practice indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
1.1	***Athlete/professional/population history:* Assesses current and past information related to personal, medical, family, and psychosocial or social history; Assesses the following:**			**X**	**X**	**X**
	1.1A	Personal history relative to age, gender self-identification, race/ethnicity, language, education, role in the family or organization, physical and/or developmental disabilities, injury, and rehabilitation		X	X	X
	1.1B	Athlete/professional medical or genetic disease states, conditions, and illnesses that may have nutritional impact		X	X	X
		1.1B1	Family history of and risk factors for medical conditions and chronic diseases (eg, diabetes, cardiovascular disease, hypertension, osteoporosis, dyslipidemia, obesity, disordered eating, behavioral or mental health issues)	X	X	X
		1.1B2	Athlete/professional history of: • metabolic and hormonal conditions (eg, diabetes, metabolic syndrome, polycystic ovary syndrome, thyroid abnormalities, exercise-induced asthma) • chronic disease clinical factors (eg, mechanical, physiological, or psychological) that may interfere with the ingestion, digestion, absorption, metabolism • utilization of nutrients • muscular/skeletal injuries (eg, sprains, stress fractures, traumatic injuries) • past and recurrent sleep disturbances, insomnia • past and current impact of stress on dietary intake • body composition • menstrual history and status	X	X	X

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Figure 1. Standards of Practice for Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance Nutrition. Note: The terms *athlete, professional, patient, client, customer, individual, person, group, organization,* or *population* are used interchangeably with the actual term used in a given situation, depending on the setting and the population receiving care or services. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry-level).

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Each RDN:				**Competent**	**Proficient**	**Expert**
		1.1B3	Effect of athlete's/professional's condition on ingestion, digestion, and absorption and metabolism of nutrients, including food allergies/intolerances (eg, gluten sensitivity or intolerance, lactose intolerance)		X	X
		1.1B4	Athlete's/professional's history of fluid or electrolyte abnormalities (eg, hyponatremia, hypernatremia)			X
		1.1B5	Athlete/professional undergoing gender transition			X
	1.1C	Athlete/professional lifestyle history (eg, use of alcohol or tobacco and recreational drugs)		X	X	X
1.2	***Anthropometric assessment:* Assesses anthropometric indicators (eg, height, weight, body mass index, waist circumference, arm circumference), comparison with reference data (eg, percentile ranks/*z*-scores), and individual patterns and history** **Assesses:**			**X**	**X**	**X**
	1.2A	Body composition/growth/weight history: Assessment measures may include: height, weight, weight change, frame size, waist circumference, growth pattern indices/percentile ranks, body compartment estimates		X	X	X
		1.2A1	Anthropometric measures, such as skinfold thickness, that require specialized training		X	X
		1.2A2	Body compartment estimates/advanced techniques (eg, densitometry [water or air displacement], bioelectrical impedance analysis, dual-energy X-ray absorptiometry)		X	X
	1.2B	Interpretation of body composition for optimal task-specific physical performance			X	X
1.3	***Biochemical data, medical tests, and procedure assessment:* Assesses laboratory profiles (eg, acid-base balance, renal function, endocrine function, inflammatory response, vitamin/mineral profile, microbiome and phytonutrient profile, lipid profile), and medical tests and procedures (eg, gastrointestinal study, metabolic rate)** **Assesses:**			**X**	**X**	**X**
	1.3A	Routine diagnostic tests and therapeutic procedures (eg, vitamin D, ferritin)		X	X	X
	1.3B	Appropriateness and recommendation for additional diagnostic tests and therapeutic procedures (eg, complete blood count comprehensive metabolic panel, endocrine markers, urinary analysis, gastric emptying study, esophagogastroduodenoscopy)			X	X

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Figure 1. *(continued)* Standards of Practice for Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance Nutrition. Note: The terms *athlete, professional, patient, client, customer, individual, person, group, organization,* or *population* are used interchangeably with the actual term used in a given situation, depending on the setting and the population receiving care or services. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry-level).

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Each RDN:			**Competent**	**Proficient**	**Expert**
	1.3C	Appropriateness and recommendation for additional specific diagnostic tests (eg, gluten intolerance, endocrine workup, malabsorption studies, sickle cell trait or disease, bone mineral density)		X	X
	1.3D	Appropriateness of tests: a) for specific populations (eg, age, sex, gender self-identification, periodized training plan, environmental conditions); b) for selection of best-fit equations in producing measurement results; and c) to manage complications (eg, pre-exercise and post-exercise glycemic control in athletes with diabetes)		X	X
1.4	***Nutrition-focused physical examination may include visual and physical examination:*** **Obtains and assesses findings from nutrition-focused physical examination (eg, indicators of vitamin or mineral deficiency or toxicity, edema, muscle wasting, subcutaneous fat loss, altered body composition, oral health, feeding ability [suck/swallow/breathe], appetite, and affect) Assesses:**		**X**	**X**	**X**
	1.4A	Clinical signs of fluid imbalance (eg, skin turgor with dehydration, fatigue, muscle cramps, dark urine, rapid weight change with fluid overload or loss, constipation)	X	X	X
	1.4B	Clinical signs of nutrition-related chronic disease (eg, acanthosis nigricans, waist circumference, body mass index)	X	X	X
	1.4C	Clinical signs of malnutrition and undernutrition (eg, lanugo, muscle wasting, dry, brittle, or thinning hair and nails)	X	X	X
	1.4D	Clinical signs of malnutrition that includes eating disorders and disordered eating (eg, hypothermia, bradycardia, lanugo, muscle wasting, tooth erosion, bony protrusions, postural orthostatic tachycardia syndrome, parotid gland enlargement, gastrointestinal distress, or dry, brittle, or thinning hair and nails)	X	X	X
1.5	***Food and nutrition—related history assessment (ie, dietary assessment)*** **Evaluates the following components:**				
	1.5A	**Food and nutrient intake, including composition and adequacy, meal and snack patterns, and appropriateness related to food allergies and intolerances Evaluates:**	**X**	**X**	**X**
		1.5A1 Adequacy of nutrition intake to maintain energy balance under various conditions (eg, recreational sports activities)	X	X	X

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Figure 1. *(continued)* Standards of Practice for Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance Nutrition. Note: The terms *athlete, professional, patient, client, customer, individual, person, group, organization,* or *population* are used interchangeably with the actual term used in a given situation, depending on the setting and the population receiving care or services. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry-level).

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Each RDN:					**Competent**	**Proficient**	**Expert**
		1.5A2	Total food and beverage intake (eg, type, amount, and pattern of intake of foods and food groups, indices of diet quality, intake of fluids), including macronutrient nutrient timing and special dietary and beverage patterns associated with recreational sport		X	X	X
			1.5A21	Total food and beverage intake (eg, type, amount, and pattern of intake of foods and food groups, indices of diet quality, intake of fluids), including nutrient timing and special dietary and beverage patterns associated with training cycle and competition or operational readiness or performance schedule		X	X
		1.5A3	Athlete/professional history of food allergies/intolerances (eg, gluten sensitivity or intolerance, lactose intolerance)		X	X	X
		1.5A4	Current and past use of nicotine, alcohol, specialized diets, sport foods or drinks, energy drinks, functional foods, liquid meal replacements, dietary supplements		X	X	X
		1.5A5	Energy balance assessment to determine energy availability for adequacy of energy intake/expenditure (eg, intake via calorie counts), including changes in body weight or composition, menstrual function, medication use, health status, or attempts at weight gain or loss		X	X	X
		1.5A6	Changes in appetite or usual intake (eg, as a result of weight control, alteration in body composition/physique, change in training volume/intensity, travel/jet lag, unfamiliar environments, training cycle and competition or operational readiness or competition schedule, medical conditions, illnesses and injuries, treatment and rehabilitation), and psychological issues (eg, stress, trauma, depression)			X	X
		1.5A7	Special energy and nutrient needs (eg, macronutrients or micronutrients) for task-specific physical performance			X	X
		1.5A8	Daily fluid needs for task-specific physical performance based on sweat rate, environmental conditions, fluid balance assessments (eg, urine specific gravity), and patterns of fluid replacement (eg, training cycle and competition or operational readiness or competition schedule)			X	X

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Each RDN:				**Competent**	**Proficient**	**Expert**
		1.5A9	Changes in usual intake as a result of dietary manipulation to optimize task-specific physical performance (eg, tapers, carbohydrate loading, glycogen restoration, rehydration, recovery nutrition after pre-competition weigh-in)		X	X
	1.5B	**Food and nutrient administration, including current and previous diets and diet prescriptions and food modifications, eating environment, and enteral and parenteral nutrition administration Evaluates:**		**X**	**X**	**X**
		1.5B1	Diet experience (eg, previously prescribed diets, previous diet or nutrition education or counseling, self-selected diets followed, dieting attempts, food allergies, food intolerances)	X	X	X
		1.5B2	Eating environment (eg, culinary skill set, location, atmosphere, caregiver or companion, eats alone)	X	X	X
		1.5B3	Nutrition issues that require medical nutrition therapy (MNT) (eg, controlled type 1 diabetes, managed gastrointestinal diseases and conditions) related to food intake and clinical complications in individuals exposed to variable training and task-specific physical performance situations		X	X
		1.5B4	Multiple or ongoing comorbidities or complications (eg, newly diagnosed type 1 diabetes, uncontrolled diabetes, recovery from injury or illness) related to food intake and clinical complications in one or more individuals or teams and their management within the interprofessional[a] treatment or performance enhancement team			X
	1.5C	**Medication and dietary supplement use, including prescription and over-the-counter medications, and integrative and functional medicine products Evaluates:**		**X**	**X**	**X**
		1.5C1	Interactions between diet and prescription and over-the-counter medications, and integrative and functional medicine products in the context of health and performance	X	X	X
		1.5C2	Safety of dietary supplements used	X	X	X
			1.5C2i Actual or potential drug/nutrient/dietary supplement interactions	X	X	X

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Figure 1. *(continued)* Standards of Practice for Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance Nutrition. Note: The terms *athlete, professional, patient, client, customer, individual, person, group, organization,* or *population* are used interchangeably with the actual term used in a given situation, depending on the setting and the population receiving care or services. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry-level).

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Each RDN:					**Competent**	**Proficient**	**Expert**
			1.5C2ii	Risk of adverse events associated with dietary supplement intake; reports all adverse events to the Food and Drug Administration's Adverse Event Reporting Program and designated medical professionals in accordance with organization policy and guidelines	X	X	X
			1.5C2iii	Dietary supplements for purity and actual or potential risk for contamination or adulteration by considering evaluations by reputable third-party independent organizations (eg, ConsumerLab.com, Natural Medicines Comprehensive Database, Natural Standard, US Pharmacopeial Convention, National Science Foundation International) that certify dietary supplements or test for banned substances	X	X	X
		1.5C3	Efficacy of dietary supplements using evidence-based practice and using other reputable resources (eg, National Institute of Health Office of Dietary Supplements, National Center for Complementary and Alternative Medicine)		X	X	X
		1.5C4	Legality of dietary supplements used		X	X	X
			1.5C4i	Compliance with Federal and state laws and regulations (eg, Drug Enforcement Agency [www.dea.gov])	X	X	X
			1.5C4ii	Compliance with sport and organization regulations and policies (eg, Department of Defense [DoD], International Olympic Committee [IOC; www.olympic.org], Major League Baseball [www.mlb.com], National Collegiate Athletic Association [NCAA; www.ncaa.org], National Federation of High School Associations [www.nfhs.org], US Anti-Doping Agency [www.usantidoping.org], US Olympic and Paralympic Committee [www.teamusa.org], World Anti-Doping Agency [WADA; www.wada-ama.org])		X	X
			1.5C4iii	Compliance with occupational regulations and policies (eg, aviation, sky diving, underwater diving)		X	X

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Figure 1. *(continued)* Standards of Practice for Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance Nutrition. Note: The terms *athlete, professional, patient, client, customer, individual, person, group, organization,* or *population* are used interchangeably with the actual term used in a given situation, depending on the setting and the population receiving care or services. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry-level).

Indicators for Standard 1: Nutrition Assessment							
Bold font indicators are Academy Core RDN Standards of Practice indicators					**The "X" signifies the indicators for the level of practice**		
Each RDN:					**Competent**	**Proficient**	**Expert**
		1.5C5	Appropriateness of dietary supplements regimen		X	X	X
			1.5C5i	Adding, maintaining, changing, or discontinuing dietary supplement use based on athlete's or professional's age, career longevity, level of competitiveness, duty status, training cycle, competitive or operational readiness or performance schedule, and travel schedule		X	X
			1.5C4ii	Dose and timing of medications and dietary supplements relative to meal timing, training cycles, competition or operational readiness, or performance schedules, travel schedules, and time zone changes		X	X
	1.5D	**Knowledge, beliefs, and attitudes (eg, understanding of nutrition-related concepts, emotions about food/nutrition/health, body image, preoccupation with food and/or weight, readiness to change nutrition- or health-related behaviors, and activities and actions influencing achievement of nutrition-related goals) Evaluates:**			**X**	**X**	**X**
		1.5D1	General food and nutrition knowledge, skills, or strategies		X	X	X
		1.5D2	Risk for or history of eating disorder or disordered eating and related factors (eg, limited food choices, distorted body image, preoccupation with food or nutrients); See Standards of Practice (SOP) and Standards of Professional Performance (SOPP) in Eating Disorders for additional information		X	X	X
		1.5D3	SHP food and nutrition knowledge, skills, or strategies			X	X
		1.5D4	Beliefs and attitudes (eg, behavioral mediators or antecedents related to SHP nutrition, intentions, readiness and willingness to change, appropriateness of goals and coping strategies)			X	X
		1.5D5	Knowledge, beliefs, or attitudes that are not evidence-based (eg, misinformation regarding nutrition for task-specific physical performance, weight management, and culture of sport or organization)			X	X
		1.5D6	Risk or history of SHP-related eating disorders and disordered eating factors such as participation in weight class, aesthetic, and weight-to-strength ratio sports and occupations with stringent height and weight standards				X

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Figure 1. *(continued)* Standards of Practice for Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance Nutrition. Note: The terms *athlete, professional, patient, client, customer, individual, person, group, organization,* or *population* are used interchangeably with the actual term used in a given situation, depending on the setting and the population receiving care or services. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry-level).

Indicators for Standard 1: Nutrition Assessment						
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Each RDN:				**Competent**	**Proficient**	**Expert**
	1.5E	**Food security defined as factors affecting access to a sufficient quantity of safe, healthful food and water, as well as food- or nutrition-related supplies Evaluates:**		**X**	**X**	**X**
		1.5E1	Presence and level of food insecurity (eg, high, marginal, low, and very low) and access to safe, healthful food and meal availability (eg, financial resources, access to farmers' markets or groceries; access to performance-based menus in institutional food service; procurement of fresh, healthful, and safe food; access to appropriate kitchen, pantry, and equipment for cooking, serving, and safe food storage)	X	X	X
		1.5E2	Access to food and nutrition related to living situation (eg, dorm, apartment, hotel, military barracks, austere deployment settings, firehouse)	X	X	X
		1.5E3	Safe water availability (eg, international travel, military operations, emergency situations, disaster events) for daily needs, including before, during, and after task-specific physical performance		X	X
	1.5F	**Physical activity, cognitive and physical ability to engage in developmentally appropriate nutrition-related tasks (eg, self-feeding and other activities of daily living [ADLs]), instrumental activities of daily living (IADLs) (eg, shopping, food preparation), and breastfeeding Evaluates:**		**X**	**X**	**X**
		1.5F1	Physical limitations affecting shopping and food preparation (eg, visual impairment, immobility)	X	X	X
		1.5F2	Availability of community resources to assist and support athlete's/professional's limitations in ADLs and IADLs		X	X
		1.5F3	Ability of organization to assist and support athlete's/professional's limitations in ADLs and IADLs			X
	1.5G	**Other factors affecting intake and nutrition and health status (eg, cultural, ethnic, religious, lifestyle influencers, psychosocial, and social determinants of health) Evaluates:**		**X**	**X**	**X**
		1.5G1	Adherence to goals (eg, self-reported adherence, visit attendance, recall of nutrition goals, self-monitoring, and self-management as agreed on)	X	X	X

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Indicators for Standard 1: Nutrition Assessment						
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Each RDN:				**Competent**	**Proficient**	**Expert**
		1.5G2	Various influences (eg, language, sport or organizational culture, ethnicity, religion) that relate to the potential for behavior change	X	X	X
		1.5G3	Sport or organization cultures that relate to the potential for behavior change		X	X
		1.5G4	Social network, social pressures, perceived social support (eg, ability to use social network, and social life, school, work influences and obligations, teammates, peers, coaches, leadership, sponsors)		X	X
1.6	***Comparative standards:* Uses reference data and standards to estimate nutrient needs and recommended body weight, body mass index, and desired growth patterns**			**X**	**X**	**X**
	1.6A	**Identifies the most appropriate reference data and/or standards (eg, international, national, state, institutional, and regulatory) based on practice setting and athlete/professional-specific factors (eg, age and disease state)**		**X**	**X**	**X**
		1.6A1	Evaluates: • energy needs, energy balance • macronutrient and micronutrient needs • fluid needs, hydration guidance, fluid balance, and electrolyte balance • weight and growth recommendations, body weight management and guidance in body weight, anthropometrics, and body composition • signs and symptoms of the female athlete triad and relative energy deficiency in sport (RED-S) • dietary supplements	X	X	X
1.7	***Physical activity habits and restrictions:* Assesses physical activity, history of physical activity, and physical activity training** **Assesses:**			**X**	**X**	**X**
	1.7A	Nontraining physical activity and exercise history and status		X	X	X
		1.7A1	History of leisure and recreational activities	X	X	X
		1.7A2	Current status of leisure and recreational activities (eg, frequency, intensity, length, and duration)	X	X	X
	1.7B	Training and performance history and status		X	X	X
		1.7B1	History of SHP training	X	X	X
		1.7B2	Current status of SHP training (eg, frequency, intensity, length, and duration)	X	X	X

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Indicators for Standard 1: Nutrition Assessment						
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Each RDN:				**Competent**	**Proficient**	**Expert**
		1.7B3	Effect of past and current dietary interventions on SHP performance outcomes, in both training and competition, operations, and performance		X	X
		1.7B4	History and current status of components of fitness (eg, maximal aerobic capacity, maximal rate of oxygen consumption, or maximal oxygen uptake, strength, power) and sport or occupation benchmark assessments (eg, National Football League [www.nfl.com] Combine, Army Physical Fitness Test, Firefighter's Work Capacity "pack" Test)		X	X
		1.7B5	Classification level of sport/occupation (eg, developmental or National Team athlete, police academy recruit or special weapons and tactics team member, conventional or special operations forces)		X	X
		1.7B6	Current training cycle and competition, operational readiness, or performance schedule		X	X
	1.7C	Limitations and special circumstances that affect nutrient intake and utilization			X	X
		1.7C1	Environmental conditions (eg, altitude training camps, operations in subtropical locations, summer wildland firefighting season)		X	X
		1.7C2	Food availability (eg, international travel, logistics chain constraints)		X	X
		1.7C3	Circadian misalignment and sleep deprivation (eg, shift work, international travel, night operations)		X	X
		1.7C4	Situation-specific stress (eg, international competition stage, combat mission, hostage negotiations, disaster relief)			X
		1.7C5	Physical disability or limitations (eg, slower gastric emptying and altered sweat response with spinal cord injury, altered thermoregulation and energy requirements with spinal cord injury and amputation)			X
1.8	**Collects data and reviews data collected and/or documented by the nutrition and dietetics technician, registered, other health care practitioner(s), athlete/professional, or staff for factors that affect nutrition and health status**			**X**	**X**	**X**
1.9	**Uses collected data to identify possible problem areas for determining nutrition diagnoses**			**X**	**X**	**X**

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Indicators for Standard 1: Nutrition Assessment					
Bold font indicators are Academy Core RDN Standards of Practice indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
1.10	**Documents and communicates:**		**X**	**X**	**X**
	1.10A	**Date and time of assessment**	**X**	**X**	**X**
	1.10B	**Pertinent data (eg, medical, social, behavioral)**	**X**	**X**	**X**
	1.10C	**Comparison with appropriate standards**	**X**	**X**	**X**
	1.10D	**Athlete/professional or population perceptions, values, and motivation related to presenting problems**	**X**	**X**	**X**
	1.10E	**Changes in athlete/professional or population perceptions, values, and motivation related to presenting problems**	**X**	**X**	**X**
	1.10F	**Reason for discharge, discontinuation, or referral, if appropriate**	**X**	**X**	**X**

Examples of Outcomes for Standard 1: Nutrition Assessment

- Appropriate assessment tools and procedures are used in valid and reliable ways
- Appropriate and pertinent data are collected
- Effective interviewing methods are used
- Data are organized and in a meaningful framework that relates to nutrition problems
- Use of assessment data leads to the determination that a nutrition diagnosis or problem does or does not exist
- Problems that require consultation with or referral to another provider are recognized
- Documentation and communication of assessment are complete, relevant, accurate, and timely

Standard 2: Nutrition Diagnosis

The registered dietitian nutritionist (RDN) identifies and labels specific nutrition problem(s) or diagnosis(es) that the RDN is responsible for treating.

Rationale:

Analysis of the assessment data leads to identification of nutrition problems and a nutrition diagnosis(es), if present. The nutrition diagnosis(es) is the basis for determining outcome goals, selecting appropriate interventions, and monitoring progress. Diagnosing nutrition problems is the responsibility of the RDN.

Indicators for Standard 2: Nutrition Diagnosis					
Bold font indicators are Academy Core RDN Standards of Practice indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
2.1	**Diagnoses nutrition problems based on evaluation of assessment data and identifies supporting concepts (ie, etiology, signs, and symptoms)**		**X**	**X**	**X**
	2.1A	Organizes and groups data consisting of intake, clinical, physical function, behavioral—environmental, and other assessments	X	X	X

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Indicators for Standard 2: Nutrition Diagnosis					
Bold font indicators are Academy Core RDN Standards of Practice indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
	2.1B	Integrates medical condition(s) (eg, hyperlipidemia, hypertension) when deriving the nutrition diagnosis(es)	X	X	X
	2.1C	Uses multiple sources of information and data (eg, biochemical, body composition, diagnostic and therapeutic procedures) obtained from assessment when deriving the nutrition diagnosis(es)		X	X
	2.1D	Compares and contrasts findings in formulating a nutrition diagnosis (eg, unintended weight loss associated with increased training volume vs purposeful weight loss via energy restriction)		X	X
2.2	**Prioritizes the nutrition problem(s)/diagnosis(es) based on severity, safety, athlete/professional needs and preferences, ethical considerations, likelihood that nutrition intervention/plan of care will influence the problem, discharge or transitions of care needs, and athlete/professional/advocate[b] perception of importance** **Uses reasoning and clinical judgment to:**		**X**	**X**	**X**
	2.2A	Select from a range of possibilities with consideration to SHP when ranking nutrition diagnoses in order of importance and urgency for the athlete/professional	X	X	X
	2.2B	Determine differences in various levels of athletes/professionals or clinical conditions (eg, newly diagnosed type 1 diabetes, uncontrolled type 1 diabetes, extreme environmental conditions, clinical complications) when ranking nutrition diagnoses in order of importance and urgency for the athlete/professional		X	X
	2.2C	Lead interprofessional team discussions to address nutrition needs and plans of care for athletes/professionals with multiple complex care issues to achieve positive outcomes			X
2.3	**Communicates the nutrition diagnosis(es) to athlete/professional advocates, community, family members, or other health care professionals when possible and appropriate**		**X**	**X**	**X**
	2.3A	Confirms and provides evidence to substantiate the nutrition diagnosis(es) with athlete/professional, family members, or other health care professionals (eg, interprofessional team) when possible and appropriate	X	X	X
	2.3B	Reports implication of nutrition diagnosis(es) to organization leadership on impact to training cycle or competition, operational readiness, or performance schedule within HIPAA limitations		X	X
2.4	**Documents the nutrition diagnosis(es) using standardized terminology and clear, concise written statement(s) (eg, using Problem [P], Etiology [E], and Signs and Symptoms [S] [PES statement(s)] or Assessment [A], Diagnosis [D], Intervention [I], Monitoring [M], and Evaluation [E] [ADIME statement(s)])**		**X**	**X**	**X**

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Indicators for Standard 2: Nutrition Diagnosis					
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Each RDN:			**Competent**	**Proficient**	**Expert**
	2.4A	Uses the electronic Nutrition Care Process Terminology (http://www.ncpro.org/) for reporting nutrition diagnosis and terminology whenever possible	X	X	X
2.5	**Reevaluates and revises nutrition diagnosis(es) when additional assessment data become available**		**X**	**X**	**X**
	2.5A	Communicates new information with nutrition implications with the athlete/professional, the interprofessional team, and other health care practitioners (eg, behavioral, medical, physical therapist), and coaches and leadership, as applicable	X	X	X

Examples of Outcomes for Standard 2: Nutrition Diagnosis

- Nutrition Diagnostic Statements that accurately describe the nutrition problem of the athlete/professional or organization in a clear and concise way
- Documentation of nutrition diagnosis(es) is relevant, accurate, and timely
- Documentation of nutrition diagnosis(es) is revised as additional assessment data become available
- Provision of relevant, accurate, and timely communication with appropriate stakeholders following HIPAA protocol

Standard 3: Nutrition Intervention/Plan of Care

The registered dietitian nutritionist (RDN) identifies and implements appropriate, person-centered interventions designed to address nutrition-related problems, behaviors, risk factors, environmental conditions, or aspects of health status for an individual, target group, or the community at large.

Rationale:

Nutrition intervention consists of two interrelated components—planning and implementation.

- Planning involves prioritizing the nutrition diagnoses, conferring with the athlete/professional and others, reviewing practice guidelines, protocols, and policies, setting goals, and defining the specific nutrition intervention strategy.
- Implementation is the action phase that includes carrying out and communicating the intervention/plan of care, continuing data collection, and revising the nutrition intervention/plan of care strategy, as warranted, based on change in condition and/or the athlete/professional response.

An RDN implements the interventions or assigns components of the nutrition intervention/plan of care to professional, technical, and support staff in accordance with knowledge, skills, and judgment, applicable laws and regulations, and organization policies. The RDN collaborates with or refers to other health care professionals and resources. The nutrition intervention or plan of care is ultimately the responsibility of the RDN.

Indicators for Standard 3: Nutrition Intervention					
Bold font indicators are Academy Core RDN Standards of Practice indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
Plans the Nutrition Intervention/Plan of Care:					
3.1	**Addresses the nutrition diagnosis(es) by determining and prioritizing appropriate interventions for the plan of care**		**X**	**X**	**X**
	Prioritization considerations may include:				
	3.1A	Immediacy of the problem	X	X	X
	3.1B	Athlete's/professional's available resources and support	X	X	X
	3.1C	Readiness of athlete/professional to receive selected nutrition interventions	X	X	X
	3.1D	Presence of medical conditions (eg, diabetes, dyslipidemia, depression, eating disorders, low bone mass, anemia, gastrointestinal conditions and disease, autoimmune disease, musculoskeletal injury)		X	X
	3.1E	Timing of the problem relative to training cycle and competition, operational readiness, or performance schedule			X
	3.1F	Prediction of potential effects of future health and performance outcomes (eg, gastrointestinal problems, dehydration, glycogen depletion, diminished mental or physical performance, nutrient or dietary supplement—drug interactions, late effects of treatments such as weight loss or gain, compromised immune system, suboptimal training adaptation)			X
3.2	**Bases intervention/plan of care on best available research or evidence and information, evidence-based guidelines, and best practices**		**X**	**X**	**X**
	3.2A	Uses professional judgment that draws from scientific literature and practice experience applicable to specific athlete's/professional's unique status and circumstances in developing an intervention plan; seeks assistance from experienced practitioner if needed	X	X	X
	3.2B	Recognizes when it is appropriate and safe to deviate from established nutrition guidelines and evidence-supported SHP practices; seeks assistance from experienced practitioner if needed		X	X
3.3	**Refers to policies and procedures, protocols, and program standards**		**X**	**X**	**X**
	3.3A	Assesses for adverse events associated with dietary supplement intake	X	X	X

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Indicators for Standard 3: Nutrition Intervention					
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Each RDN:			**Competent**	**Proficient**	**Expert**
	3.3B	Complies with sport and organizational guidance, regulations, and policies regarding food and nutrition and dietary supplements (eg, DoD, IOC, Major League Baseball [www.mlb.com], NCAA, National Federation of High School Associations [www.nfhs.org], US Anti-Doping Agency [www.usantidoping.org], US Olympic and Paralympic Committee [www.teamusa.org], World Anti-Doping Agency [www.wada-ama.org])		X	X
	3.3C	Educates athletes/professionals and others regarding applicable organization guidance and regulations concerning use of dietary supplements, including banned or prohibited substances for specific athletic and tactical populations (eg, NCAA, Olympic and professional athletes, and DoD service members)		X	X
3.4	**Collaborates with athlete/professional, advocate, or population, caregivers, interprofessional team, and other health care professionals**		**X**	**X**	**X**
	3.4A	Recognizes specific knowledge and skills of interprofessional team, and collaborates to provide comprehensive care	X	X	X
	3.4B	Provides rationales for nutrition interventions to colleagues and interprofessional team members		X	X
3.5	**Works with athlete/professional, advocate, or organization to identify goals, preferences, discharge or transitions of care needs, plan of care, and expected outcomes**		**X**	**X**	**X**
	3.5A	Conducts thorough analysis of athlete's/professional's expectations in context of organization competition, operational readiness, or performance schedule		X	X
3.6	**Develops the nutrition prescription and establishes measurable athlete-/professional—focused goals to be accomplished**		**X**	**X**	**X**
	3.6A	Considers the educational needs of the athlete/professional, including cultural competency	X	X	X
	3.6B	Plans intervention to address current issues (eg, fatigue, dehydration, muscle cramping, inadequate recovery, exercise, task-specific physical performance improvement and training adaptation, diarrhea and other gastrointestinal issues, illness, injury, disordered eating, female athlete triad; RED-S, dietary supplement use, or other clinical issues)	X	X	X
	3.6C	Selects intervention strategies that are based on evidence, best practices, and professional experience	X	X	X

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Indicators for Standard 3: Nutrition Intervention						
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Each RDN:				**Competent**	**Proficient**	**Expert**
		3.6C1	Identifies barriers to successful implementation (eg, athlete/professional compliance, time constraints, food availability, storage, and culinary skill set, financial issues, regulations of sport governing bodies and associations, organizational culture and policies, situations in occupational settings, athlete/professional influencer dynamics)	X	X	X
		3.6C2	Balances short-term task-specific physical performance goals with long-term career longevity goals and outcomes		X	X
	3.6D	Addresses issues related to off-season or transition body composition changes, detraining, and scheduled and unscheduled breaks in training such as holidays, post-deployment military block leave, and extended travel			X	X
	3.6E	Develops and implements strategies to minimize barriers and other issues, using concepts of the three components of the SHP program model (eg, timing of intervention relative to annual training and competition, operational readiness, or performance schedule and travel plans; prevent treatment delays; reduce relapse; and reduce the need for more advanced/involved treatment options)			X	X
3.7	**Defines time and frequency of care, including intensity, duration, and follow-up**			**X**	**X**	**X**
	3.7A	Identifies time and frequency for ordering and monitoring results of diagnostic tests or procedures and laboratory tests, based on athlete/professional needs, established goals and outcomes, and expected response to intervention(s), reflecting organization or program policies or regulations when applicable			X	X
	3.7B	Considers organization's talent development and longitudinal health and performance surveillance strategies when ordering laboratory tests and procedures				X
3.8	**Uses standardized terminology for describing interventions**			**X**	**X**	**X**
	3.8A	Uses standardized electronic Nutrition Care Process Terminology, when applicable, and other SHP terms to describe interventions			X	X
3.9	**Identifies resources and referrals needed**			**X**	**X**	**X**
	3.9A	Collaborates with and refers to qualified RDNs for complex cases (eg, eating disorders, nutrition support, diabetes, oncology)		X	X	X
	3.9B	Identifies resources to assist athlete/professional in using available resources (eg, meal preparation or delivery services, websites)		X	X	X

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Indicators for Standard 3: Nutrition Intervention					
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Each RDN:			**Competent**	**Proficient**	**Expert**
	3.9C	Understands the role of various disciplines (eg, strength and conditioning, physical therapy, athletic training, cognitive performance, foodservice) to facilitate appropriate referrals as needed	X	X	X
Implements the Nutrition Intervention or Plan of Care:					
3.10	**Collaborates with colleagues, interprofessional team, and other health care professionals**		**X**	**X**	**X**
	3.10A	Participates in communications within the interprofessional team or performance enhancement team	X	X	X
	3.10B	Facilitates and fosters active communication, learning partnerships, and collaboration with the interprofessional team or with other providers or consultants within the SHP program concept	X	X	X
	3.10C	Seeks opportunities to collaborate and share information that supports the integration of SHP nutrition with other discipline interventions, particularly when results are not being achieved (eg, expected training adaptations or rehabilitation outcomes)		X	X
	3.10D	Identifies and seeks opportunities for external and interagency collaboration, specific to the athlete's/professional's needs			X
3.11	**Communicates and coordinates the nutrition intervention/plan of care**		**X**	**X**	**X**
	3.11A	Communicates nutrition intervention/plan of care with athlete/professional	X	X	X
	3.11B	Communicates and coordinates nutrition intervention/plan of care with appropriate members of the interprofessional team or performance enhancement team, particularly for awareness of potentially conflicting or problematic interventions (eg, increased energy intake to facilitate healing process and decreased need for energy intake because of injury-induced decreased training volume or intensity)		X	X
	3.11C	Coordinates nutrition intervention/plan of care with training cycle and competition, operational readiness, and performance schedule and with rehabilitation during recovery from illness or injury		X	X
	3.11D	Communicates nutrition intervention/plan of care with coaches and leadership within regulatory limitations; seeks support when needed		X	X
3.12	**Initiates and individualizes the nutrition intervention/plan of care**		**X**	**X**	**X**
	3.12A	**Uses approved clinical privileges, physician or non-physician practitioner[c]—driven orders (ie, delegated orders), protocols, or other facility-specific processes for order writing or for provision of nutrition-related services consistent with applicable specialized training, competence, medical staff, and/or organizational policy**	**X**	**X**	**X**

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Figure 1. *(continued)* Standards of Practice for Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance Nutrition. Note: The terms *athlete, professional, patient, client, customer, individual, person, group, organization,* or *population* are used interchangeably with the actual term used in a given situation, depending on the setting and the population receiving care or services. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry-level).

Indicators for Standard 3: Nutrition Intervention							
Bold font indicators are Academy Core RDN Standards of Practice indicators					**The "X" signifies the indicators for the level of practice**		
Each RDN:					**Competent**	**Proficient**	**Expert**
		3.12A1	**Implements, initiates, or modifies orders for therapeutic diet, nutrition-related pharmacotherapy management, or nutrition-related services (eg, food texture modifications, intravenous fluid infusions, laboratory tests, and medications)**		**X**	**X**	**X**
			3.12A1i	Uses physician- or referring practitioner —driven protocols to manage the use of dietary supplements, sports and functional foods (eg, portion or dosage adjustments based on energy needs, anthropometric data, or laboratory results; addition of new dietary supplements, sports foods, functional foods; or modifications for fluid intake) consistent with organization policy, if applicable	X	X	X
		3.12A2	**Manages nutrition therapies (eg, medical foods, dietary supplements [vitamins and minerals], and oral nutrition supplements); collaborates with or refers to an applicable expert RDN when appropriate**		**X**	**X**	**X**
		3.12A3	**Initiates and performs nutrition-related services (eg, education, health and wellness coaching, and counseling)**		**X**	**X**	**X**
	3.12B	Uses appropriate goal setting and behavior change strategies and techniques to facilitate self-management and self-care			X	X	X
	3.12C	Uses critical thinking and synthesis skills to guide decision making in routine situations integrating nutrition with task-specific physical performance				X	X
	3.12D	Uses critical thinking and synthesis skills to guide decision-making in MNT issues (eg, controlled type 1 diabetes, managed gastrointestinal conditions and diseases)				X	X
	3.12E	Uses critical thinking and synthesis skills to guide decision-making in multiple or unmanaged MNT issues, and dynamic situations (eg, newly diagnosed type 1 diabetes, uncontrolled diabetes, extreme environmental conditions, clinical complications)					X
3.13	**Assigns activities to nutrition and dietetics technician, registered, and other professional, technical, and support personnel in accordance with qualifications, organization policies or protocols, and applicable laws and regulations**				**X**	**X**	**X**
	3.13A	**Supervises support personnel**			**X**	**X**	**X**

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Figure 1. *(continued)* Standards of Practice for Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance Nutrition. Note: The terms *athlete, professional, patient, client, customer, individual, person, group, organization,* or *population* are used interchangeably with the actual term used in a given situation, depending on the setting and the population receiving care or services. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry-level).

Indicators for Standard 3: Nutrition Intervention					
Bold font indicators are Academy Core RDN Standards of Practice indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
	3.13B	Provides professional, technical, and support personnel with information and guidance needed to complete assigned activities (eg, collecting and reporting anthropometric data, purchase and distribution of dietary supplements, or managing fueling stations)		X	X
3.14	**Continues data collection**		**X**	**X**	**X**
	3.14A	Identifies and records specific data collection for athlete/professional, including weight change, biochemical, behavioral, and lifestyle factors, using prescribed and standardized format	X	X	X
	3.14B	Identifies and records specific data collection for athlete/professional, including training cycle, and competition, operational readiness, or performance schedule, using prescribed, standardized format		X	X
3.15	**Documents:**				
	3.15A	**Date and time**	**X**	**X**	**X**
	3.15B	**Specific and measurable treatment goals and expected outcomes**	**X**	**X**	**X**
	3.15C	**Recommended interventions**	**X**	**X**	**X**
	3.15D	**Athlete/professional/advocate/caregiver/community receptiveness**	**X**	**X**	**X**
	3.15E	**Referrals made and resources used**	**X**	**X**	**X**
	3.15F	**Athlete/professional, advocate, caregiver, and community comprehension**	**X**	**X**	**X**
	3.15G	**Barriers to change**	**X**	**X**	**X**
	3.15H	**Other information relevant to providing care and monitoring progress over time**	**X**	**X**	**X**
	3.15I	**Plans for follow-up and frequency of care**	**X**	**X**	**X**
	3.15J	**Rationale for discharge or referral, if applicable**	**X**	**X**	**X**
	3.15K	**Specific and measurable treatment goals and expected outcomes**	**X**	**X**	**X**

Examples of Outcomes for Standard 3: Nutrition Intervention/Plan of Care

- Goals and expected outcomes are appropriate and prioritized
- Athlete/professional, advocate, population, caregivers, and interprofessional teams collaborate and are involved in developing nutrition intervention/plan of care

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Figure 1. *(continued)* Standards of Practice for Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance Nutrition. Note: The terms *athlete, professional, patient, client, customer, individual, person, group, organization,* or *population* are used interchangeably with the actual term used in a given situation, depending on the setting and the population receiving care or services. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry-level).

- Appropriate individualized athlete-/professional-centered nutrition intervention/plan of care, including nutrition prescription, is developed
- Nutrition intervention/plan of care is delivered, and actions are carried out as intended
- Discharge planning and transitions of care needs are identified and addressed
- Documentation of nutrition intervention/plan of care is:
  - Specific
  - Measurable
  - Attainable
  - Relevant
  - Timely
  - Comprehensive
  - Accurate
  - Dated and timed

Standard 4: Nutrition Monitoring and Evaluation
The registered dietitian nutritionist (RDN) monitors and evaluates indicators and outcomes data directly related to the nutrition diagnosis, goals, preferences, and intervention strategies, to determine the progress made in achieving desired results of nutrition care and whether planned interventions should be continued or revised.
Rationale:
Nutrition monitoring and evaluation are essential components of an outcomes management system to assure quality athlete/professional- or population-centered care and to promote uniformity within the profession in evaluating the efficacy of nutrition interventions. Through monitoring and evaluation, the RDN identifies important measures of change or outcomes relevant to the nutrition diagnosis and nutrition intervention/plan of care; describes how best to measure these outcomes; and intervenes when the intervention/plan of care requires revision.

Indicators for Standard 4: Nutrition Monitoring and Evaluation						
Bold font indicators are Academy Core RDN Standards of Practice indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
4.1	**Monitors progress:**			**X**	**X**	**X**
	4.1A	**Assesses athlete/professional understanding and compliance with nutrition intervention/plan of care**		**X**	**X**	**X**
		4.1A1	Assesses compliance with recommended energy, fluid, and dietary supplement amounts and timing	X	X	X
	4.1B	**Determines whether the nutrition intervention/plan of care is being implemented as prescribed**		**X**	**X**	**X**
		4.1B1	Evaluates implementation of nutrition intervention plan in all levels of athletes/professionals relative to training and task-specific physical performance issues; seeks assistance if needed	X	X	X

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Figure 1. *(continued)* Standards of Practice for Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance Nutrition. Note: The terms *athlete, professional, patient, client, customer, individual, person, group, organization,* or *population* are used interchangeably with the actual term used in a given situation, depending on the setting and the population receiving care or services. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry-level).

Indicators for Standard 4: Nutrition Monitoring and Evaluation						
Bold font indicators are Academy Core RDN Standards of Practice indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
		4.1B2	Evaluates implementation of nutrition intervention plan in athletes/professionals balancing multiple situations (eg, environmental extremes, rapid weight changes, travel, sporting events, disaster or emergency situations, military operations, and/or clinical complications); seeks assistance from experienced practitioner if needed		X	X
		4.1B3	Evaluates implementation of nutrition intervention plan in athletes/professionals with MNT issues (eg, controlled diabetes, managed gastrointestinal conditions and diseases)		X	X
4.2	**Measures outcomes:**			**X**	**X**	**X**
	4.2A	**Selects the standardized nutrition care measurable outcome indicator(s) Considers:**		**X**	**X**	**X**
		4.2A1	Physical measures I (eg, weight maintenance, body weight, anthropometric measures, body composition changes, laboratory values)	X	X	X
		4.2A2	Physical measures II (eg, fluid and electrolyte balance, bone density) relative to physical and task-specific physical performance indicators		X	X
		4.2A3	Behavioral and treatment outcomes (eg, minimize barriers, prevent treatment delays, reduce relapse, and the need for more advanced or involved treatment options)		X	X
		4.2A4	Quality of life and task-specific physical performance indicators (eg, full participation in sport or work assignment, improved mood and cognitive function, progress in rehabilitation, minimization of fatigue, absence of muscle cramping, avoidance of gastrointestinal problems)			X
	4.2B	**Identifies positive or negative outcomes, including impact on potential needs for discharge or transitions of care**		**X**	**X**	**X**
		4.2B1	Documents progress in meeting desired goals (eg, energy balance, fluid, nutrient intake, body weight, anthropometric measures, body composition, overall performance)	X	X	X
		4.2B2	Determines whether outcomes meet expectations		X	X
		4.2B3	Identifies unintended consequences (eg, not meeting set weight class goal for competition) or the use of inappropriate methods of achieving goals			X

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Indicators for Standard 4: Nutrition Monitoring and Evaluation						
Bold font indicators are Academy Core RDN Standards of Practice indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
4.3	**Evaluates outcomes:**			**X**	**X**	**X**
	4.3A	**Compares monitoring data with nutrition prescription and established goals or reference standard**		**X**	**X**	**X**
		4.3A1	Uses athlete's/professional's current and historical data and trends to evaluate against reference standards		X	X
	4.3B	**Evaluates impact of the sum of all interventions on overall** athlete/professional **or population health outcomes and goals**		**X**	**X**	**X**
		4.3B1	Evaluates sum of all interventions against overall organization outcomes and goals		X	X
	4.3C	**Evaluates progress or reasons for lack of progress related to problems and interventions**		**X**	**X**	**X**
		4.3C1	Uses multiple data sources to evaluate progress; examples may include: • nutrition-focused physical examination, including but not limited to signs of fluid, energy, or nutrition depletion or excess • adequacy of energy and nutrient intake from all sources • fluid and electrolyte balance • gastrointestinal tolerance • changes in body weight, anthropometric measures, body composition • signs and symptoms of disordered eating, the female athlete triad, and RED-S Seeks assistance if needed	X	X	X
		4.3C2	Uses multiple data sources to evaluate progress in more complex situations; examples may include: • pertinent medications and dietary supplements and implementation of nutrition intervention • effects of intervention on task-specific physical performance and recovery • effects of intervention on progress in rehabilitation and recovery from injury		X	X
		4.3C3	Considers organization's culture, policies, and procedures (eg, organization cultural aversion to seeking follow-up care, lack of organization support for dedicated time required for intervention)			X

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Indicators for Standard 4: Nutrition Monitoring and Evaluation						
Bold font indicators are Academy Core RDN Standards of Practice indicators				The "X" signifies the indicators for the level of practice		
Each RDN:				Competent	Proficient	Expert
	4.3D	**Evaluates evidence that the nutrition intervention/plan of care is maintaining or influencing a desirable change in the** athlete/professional **or population behavior or status**		X	X	X
		4.3D1	Monitors and interprets laboratory and other data that may reflect a change in the athlete's/professional's behavior or status	X	X	X
		4.3D2	Monitors factors (eg, physical, social, cognitive, environmental) that may reflect a change affected by the nutrition intervention	X	X	X
	4.3E	**Supports conclusions with evidence**		X	X	X
		4.3E1	Documents processes and outcomes using PES statements	X	X	X
4.4	**Adjusts nutrition intervention/plan of care strategies, if needed, in collaboration with** athlete/professional**, population, advocate, or caregiver and interprofessional team**			X	X	X
	4.4A	**Improves or adjusts intervention/plan of care strategies based on outcomes data, trends, best practices, and comparative standards**		X	X	X
	4.4B	Modifies nutrition intervention based on athlete/professional tolerance, response, environmental limitations, changes to training cycle, and competition, operational readiness, or performance schedule, and outcome measures with regard to documented goals and objectives			X	X
	4.4C	Adjusts intervention strategies by drawing on experience, clinical judgment, and research- or evidence-based practice guidelines about the athlete/professional populations in complicated and unpredictable situations (eg, pregnancy, eating disorders, cancer)				X
4.5	**Documents:**			X	X	X
	4.5A	**Date and time**		X	X	X
	4.5B	**Indicators measured, results, and the method for obtaining measurement**		X	X	X
	4.5C	**Criteria with which the indicator is compared (eg, nutrition prescription or goal or a reference standard)**		X	X	X
	4.5D	**Factors facilitating or hampering progress**		X	X	X
	4.5E	**Other positive or negative outcomes**		X	X	X
	4.5F	**Adjustments to the nutrition intervention/plan of care, if indicated**		X	X	X
	4.5G	**Future plans for nutrition care, nutrition monitoring and evaluation, follow-up, referral, or discharge**		X	X	X

Examples of Outcomes for Standard 4: Nutrition Monitoring and Evaluation

- The athlete/professional outcome(s) directly relate to the nutrition diagnosis and the goals established in the nutrition intervention/plan of care. Examples include, but are not limited to:
  - Nutrition outcomes (eg, change in knowledge, behavior, food, or nutrient intake)
  - Clinical and health status outcomes (eg, change in laboratory values, body weight, blood pressure, risk factors, signs and symptoms, clinical status, infections, complications, morbidity, and mortality)
  - Athlete-/professional-centered outcomes (eg, quality of life, satisfaction, self-efficacy, self-management, functional ability)
- Nutrition intervention/plan of care and documentation is revised, if indicated
- Documentation of nutrition monitoring and evaluation is:
  - Specific
  - Measurable
  - Attainable
  - Relevant
  - Timely
  - Comprehensive
  - Accurate
  - Dated and timed

[a]**Interprofessional:** The term *interprofessional* is used in this evaluation resource as a universal term. It includes a diverse group of team members (eg, physicians, nurses, dietitian nutritionists, pharmacists, psychologists, social workers, strength and conditioning coaches, certified athletic trainers, and occupational and physical therapists), depending on the needs of the athlete/professional. Interprofessional could also mean interdisciplinary or multidisciplinary.

[b]**Advocate:** An *advocate* is a person who provides support and/or represents the rights and interests at the request of the athlete/professional. The person may be a family member or an individual not related to the athlete/professional who is asked to support the athlete/professional with activities of daily living or is legally designated to act on behalf of the athlete/professional, particularly when the athlete/professional has lost decision-making capacity. (Adapted from definitions within The Joint Commission Glossary of Terms[15] and the Centers for Medicare and Medicaid Services, Hospital Conditions of Participation[6]).

[c]**Non-physician practitioner**: A *nonphysician practitioner* may include a physician assistant, nurse practitioner, clinical nurse specialist, certified registered nurse anesthetist, certified nurse-midwife, clinical social worker, clinical psychologist, anesthesiologist's assistant, qualified dietitian or qualified nutrition professional, occupational and physical therapists, or qualified nutrition professional. Disciplines considered for privileging by a facility's governing body and medical staff must be in accordance with state law.[6,7]

Figure 1. *(continued)* Standards of Practice for Registered Dietitian Nutritionists (RDNs) in Sports and Human Performance Nutrition. Note: The terms *athlete, professional, patient, client, customer, individual, person, group, organization,* or *population* are used interchangeably with the actual term used in a given situation, depending on the setting and the population receiving care or services. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry-level).

Standards of Professional Performance for Registered Dietitian Nutritionists in Sports and Human Performance Nutrition
Standard 1: Quality in Practice
The registered dietitian nutritionist (RDN) provides quality services using a systematic process with identified ethics, leadership, accountability, and dedicated resources.
Rationale:
Quality practice in nutrition and dietetics is built on a solid foundation of education and supervised practice, credentialing, evidence-based practice, demonstrated competence, and adherence to established professional standards. Quality practice requires systematic measurement of outcomes, regular performance evaluations, and continuous improvement.

Indicators for Standard 1: Quality in Practice					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
1.1	**Complies with applicable laws and regulations as related to their area(s) of practice**		**X**	**X**	**X**
	1.1A	Complies with Health Insurance Portability and Accountability Act (HIPAA) and organization's regulations and standards regarding sharing of protected health information and personally identifiable information	X	X	X
	1.1B	Complies with state licensure or certification laws and regulations, if applicable, including telehealth and continuing education requirements	X	X	X
	1.1C	Addresses dietary supplement products to ensure compliance with anti-doping rules and regulations of sports organizations and governing bodies (eg, International Olympic Committee [IOC; www.olympic.org], International Paralympic Committee [IPC; www.paralympic.org], National Collegiate Athletic Association [NCAA], National Federation of High School Associations [NFHS; www.nfhs.org], US Anti-Doping Agency [USADA; www.usantidoping.org], US Olympic and Paralympic Committee [USOPC; http://www.teamusa.org/], World Anti-Doping Agency [WADA; www.wda-ama.org])		X	X
1.2	**Performs within individual and statutory scope of practice and applicable laws and regulations**		**X**	**X**	**X**
	1.2A	Demonstrates competence to practice within job description and contract, and maintains qualifications, as outlined in contractual statements of work or position description	X	X	X
	1.2B	Follows any scope of practice requirements related to additional credentialing or position (eg, Certified Specialist in Sport Dietetics [CSSD], Certified Strength and Conditioning Coach [CSCC])		X	X

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Figure 2. Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 1: Quality in Practice						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
1.3	**Adheres to sound business and ethical billing practices applicable to the role and setting**			**X**	**X**	**X**
	1.3A	Assures ethical and accurate reporting of SHP nutrition services (eg, billing codes) and compliance with contracts or funder requirements, when applicable.		X	X	X
1.4	**Uses national quality and safety data (eg, National Academies of Sciences, Engineering, and Medicine: Health and Medicine Division, National Quality Forum, Institute for Healthcare Improvement) to improve the quality of services provided and to enhance customer-centered services**			**X**	**X**	**X**
1.5	**Uses a systematic performance improvement model that is based on practice knowledge, evidence, research, and science for delivery of the highest quality services**			**X**	**X**	**X**
	1.5A	Identifies performance improvement criteria to monitor effectiveness of services		X	X	X
	1.5B	Assists in designing performance improvement programs that use evidence-based protocols to evaluate effectiveness of services			X	X
	1.5C	Develops implementation strategies and leads quality improvement activities (eg, identification and adaptations of evidence-based practice guidelines and protocols, skills training or reinforcement, organization support and incentives)				X
	1.5D	Assures completeness of organization testing protocols (ie, proactive, active, and reactive component relevant tests) within regulatory standards and for integration with other multidisciplinary testing				X
1.6	**Participates in or designs an outcomes-based management system to evaluate safety, effectiveness, quality, person-centeredness, equity, timeliness, and efficiency of practice**			**X**	**X**	**X**
	1.6A	**Involves colleagues and others, as applicable, in systematic outcomes management**		**X**	**X**	**X**
		1.6A1	Participates in and/or uses collected data as part of a quality improvement process relative to outcomes, quality of care, and services rendered	X	X	X
		1.6A2	Engages organization leadership in developing and monitoring outcomes-based management systems		X	X
		1.6A3	Engages organization data analysts to evaluate both relative and absolute differences of outcomes data between different subpopulations to monitor program equity			X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 1: Quality in Practice						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
	1.6B	**Defines expected outcomes**		**X**	**X**	**X**
		1.6B1	Considers available resources (eg, time, personnel, infrastructure) when developing expected outcomes		X	X
		1.6B2	Considers organization mission, vision, objectives, and values when defining expected outcomes			X
	1.6C	**Uses indicators that are specific, measurable, attainable, realistic, and timely (S.M.A.R.T.)**		**X**	**X**	**X**
		1.6C1	Identifies and promotes use of evidence-based evaluation criteria applicable to SHP nutrition	X	X	X
		1.6C2	Relates program outcomes to multilevel outcomes (eg, organization, program, and/or individual outcomes or needs)		X	X
	1.6D	**Measures quality of services in terms of structure, process, and outcomes**		**X**	**X**	**X**
		1.6D1	Collects data to evaluate, improve, and document outcomes and services	X	X	X
		1.6D2	Practices in accordance with the goals and objectives of continuous quality improvement	X	X	X
		1.6D3	Considers short-, medium-, and long-term outcomes, including cost-effectiveness, collaborating with interprofessional[a] team and others		X	X
		1.6D4	Initiates and/or facilitates the development and evaluation of processes and outcomes		X	X
		1.6D5	Monitors and evaluates data against expected outcomes; adjusts processes based on results		X	X
		1.6D6	Leads in educating and mentoring practitioners in measuring SHP nutrition processes to determine effectiveness			X
		1.6D7	Ensures quality of service measures are aligned with organization mission, vision, and objectives			X
		1.6D8	Incorporates SHP nutrition-relevant return on investment measures into evaluation of processes and outcomes			X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 1: Quality in Practice						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
	1.6E	**Incorporates electronic clinical quality measures to evaluate and improve care of** athlete/professional **at risk for malnutrition or with malnutrition (www.eatrightpro.org/emeasures)**		**X**	**X**	**X**
		1.6E1	Ensures that screening for nutrition risk (eg, anemia, relative energy deficiency in sport) is a component of program admission process or nutrition assessment using evidence-based screening tools for the setting or population	X	X	X
		1.6E2	Collects data using quality measures applicable to population and setting (eg, screening timeframes, number at risk or with malnutrition, and services provided [eg, nutrition assessment, nutrition and/or dietary supplements, nutrition counseling])		X	X
	1.6F	**Documents outcomes and patient-reported outcomes (eg, PROMIS[b])**		**X**	**X**	**X**
		1.6F1	Documents objective and subjective outcomes in accordance with organization policies and standards	X	X	X
	1.6G	**Participates in, coordinates, or leads program participation in local, regional, or national registries and data warehouses used for tracking, benchmarking, and reporting service outcomes**		**X**	**X**	**X**
		1.6G1	Directs the development, monitoring, and evaluation of practice-specific benchmarks (eg, appropriate hydration practices, body weight and body composition management strategies) relevant to national initiatives (eg, Academy, Dietitians of Canada, American College of Sport Medicine [ACSM; http://www.acsm.org], NCAA, sports organizations and governing bodies, and occupational organizations) to improve outcomes		X	X
	1.6H	Advocates for and participates in developing a system for clinical, operational, and financial data collection and analysis on which outcomes can be derived, reported, and used for improvement			X	X
1.7	**Identifies and addresses potential and actual errors and hazards in provision of services or brings to attention of supervisors and team members as appropriate**			**X**	**X**	**X**
	1.7A	Recognizes potential drug—nutrient interactions, dietary supplement safety and interactions, and potential interactions between interventions and other therapies as potential hazards; provides education and counseling as appropriate		X	X	X

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Indicators for Standard 1: Quality in Practice					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
	1.7B	Evaluates dietary supplement product and manufacturing practices, quality control, error prevention recommendations (eg, as provided by The Institute for Safe Medication Practices [www.ismp.org], the Food and Drug Administration, the US Pharmacopeial Convention [www.usp.org])	X	X	X
	1.7C	Reports dietary supplement adverse events to the Food and Drug Administration's Adverse Event Reporting System and appropriate medical provider per organization guidelines	X	X	X
	1.7D	Addresses and documents the potential for errors and hazards, and implements corrective or preventive measures (eg, education programs, system alerts) with periodic audits or according to organization policy; works with facility staff to reduce errors and hazards		X	X
	1.7E	Reviews dietary supplement products to ensure compliance with organization drug abuse programs, policies, and guidelines		X	X
	1.7F	Develops protocols to identify, address, and prevent errors and hazards in the delivery of SHP nutrition services			X
1.8	**Compares actual performance to performance goals (ie, gap analysis, SWOT analysis [strengths, weaknesses, opportunities, and threats], PDCA Cycle [plan-do-check-act], DMAIC [define, measure, analyze, improve, control])**		**X**	**X**	**X**
	1.8A	Reports and documents action plan to address identified gaps in care and/or service performance	X	X	X
	1.8B	Compares individual performance with self-directed goals and expected outcomes and improvement recommendations	X	X	X
	1.8C	Compares department or organization performance with goals and expected outcomes to identify improvement recommendations or actions in collaboration with interprofessional team and other stakeholders		X	X
	1.8D	Benchmarks department or organization performance with national programs and standards		X	X
1.9	**Evaluates interventions and workflow process(es) and identifies service and delivery improvements**		**X**	**X**	**X**
	1.9A	Engages athletes/professionals in intervention evaluations (eg, athlete/professional satisfaction surveys) to identify service and delivery improvements	X	X	X
	1.9B	Applies performance improvement and research data to SHP nutrition practice to improve effectiveness and efficiency		X	X
	1.9C	Designs and implements nutrition evaluation protocols, analyzes data, and implements improvements			X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 1: Quality in Practice					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
1.10	**Improves or enhances athlete/professional or population care and/or services, working with others based on measured outcomes and established goals**		**X**	**X**	**X**
	1.10A	Evaluates and improves professional performance in relation to established outcomes of quality in SHP nutrition services	X	X	X
	1.10B	Assesses outcomes to identify areas that need improvement in SHP nutrition services	X	X	X
	1.10C	Directs the development and management of systems, processes, and programs that advance best practices and the core values and objectives of SHP nutrition services		X	X
	1.10D	Leads local, state, national, or international quality initiative efforts to support goals and best practices of SHP nutrition			X

Examples of Outcomes for Standard 1: Quality in Practice

- Actions are within scope of practice and applicable laws and regulations
- National quality standards and best practices are evident in customer-centered services
- Performance improvement systems specific to program(s) or service(s) are established and updated as needed; are evaluated for effectiveness in providing desired outcomes data and striving for excellence in collaboration with other team members
- Performance indicators are specific, measurable, attainable, realistic, and timely (S.M.A.R.T.)
- Aggregate outcomes results meet preestablished criteria
- Quality improvement results direct refinement and advancement of practice

Standard 2: Competence and Accountability

The registered dietitian nutritionist (RDN) demonstrates competence in and accepts accountability and responsibility for ensuring safe, quality practice and services.

Rationale:

Competence and accountability in practice includes continuous acquisition of knowledge, skills, experience, and judgment in the provision of safe, quality customer-centered service.

Indicators for Standard 2: Competence and Accountability					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
2.1	**Adheres to the code(s) of ethics (eg, Academy/Commission on Dietetic Registration [CDR], other national organizations, and/or employer code of ethics)**		**X**	**X**	**X**

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 2: Competence and Accountability						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
	2.1A	Discloses any financial relationships regarding the sale or recommendation of food, beverages, and/or dietary supplements to athlete/professional or organization		X	X	X
	2.1B	Informs athlete/professional or organization on the availability of products being recommended as well as equivalent products on the market		X	X	X
	2.1C	Ensures compliance with organization policies regarding contact with industry in all vendor interactions			X	X
	2.1D	Considers organization's sponsorship or endorsement exclusivity agreements				X
2.2	**Integrates the Standards of Practice (SOP) and Standards of Professional Performance (SOPP) into practice, self-evaluation, and professional development**			**X**	**X**	**X**
	2.2A	**Integrates applicable focus area(s) SOP and/or SOPP into practice (www.eatrightpro.org/sop)**		**X**	**X**	**X**
		2.2A1	Uses the SOP and SOPP in SHP nutrition to: • assess performance at the appropriate level of practice • develop and implement a professional development plan to enhance practice and performance • develop and implement a professional development plan to advance practice and performance to a higher level	X	X	X
		2.2A2	Develops and/or maintains department or organization policy, guidelines, resource materials (eg, position description or agency nutrition service performance standards, career ladders, acceptable performance level) reflecting the SOP and SOPP for RDNs in SHP Nutrition and evidence-based research and best practices		X	X
		2.2A3	Develops performance criteria for employment setting and for nutrition programs within local, state, and/or federal agencies, reflecting levels of practice described in the SOP and SOPP for RDNs in SHP Nutrition			X
2.3	**Demonstrates and documents competence in practice and delivery of customer-centered service(s)**			**X**	**X**	**X**
	2.3A	Seeks out qualified peers to demonstrate and document competency		X	X	X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 2: Competence and Accountability						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
2.4	**Assumes accountability and responsibility for actions and behaviors**			**X**	**X**	**X**
	2.4A	**Identifies, acknowledges, and corrects errors**		**X**	**X**	**X**
	2.4B	Recognizes and informs athlete/professional or organization of strengths and limitations of current information, research, and evidence when making recommendations; seeks assistance if needed		X	X	X
	2.4C	Evaluates RDNs in SHP nutrition performance based on level of education, skills, and performance requirements when in supervisory or management position			X	X
	2.4D	Develops and implements policies and procedures that ensure staff accountability and responsibility			X	X
2.5	**Conducts self-assessment at regular intervals**			**X**	**X**	**X**
	2.5A	**Identifies needs for professional development**		**X**	**X**	**X**
		2.5A1	Uses self-assessment tools to evaluate personal and professional factors (eg, personality type, communication style, management style)	X	X	X
		2.5A2	Uses self-assessment tools to evaluate SHP nutrition professional knowledge, skill, and practice consistent with best practices and research findings according to level of practice (eg, Academy's Scope of Practice Decision Algorithm)	X	X	X
		2.5A3	Engages in self-assessment to ascertain progress in meeting desired performance outcomes	X	X	X
	2.5B	Seeks opportunities for professional development by reviewing evidence-based guidelines, and current research findings, consistent with identified needs and career goals		X	X	X
	2.5C	Seeks opportunities for multi-source feedback when in supervisory or management positions			X	X
2.6	**Designs and implements plans for professional development**			**X**	**X**	**X**
	2.6A	**Develops plan and documents professional development activities in career portfolio (eg, organizational policies and procedures, credentialing agency[ies])**		**X**	**X**	**X**
		2.6A1	Documents, in professional development plan, activities that demonstrate professional responsibilities at the appropriate level of practice	X	X	X
		2.6A2	Reviews current literature and education materials specific to SHP nutrition (eg, peer-reviewed articles, textbook chapters, books, podcasts, and webinars from content experts); consults with RDN experienced in SHP nutrition as needed	X	X	X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 2: Competence and Accountability					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
2.7	**Engages in evidence-based practice and uses best practices**		**X**	**X**	**X**
	2.7A	Uses evidence, including research, national guidelines, policies, consensus statements, systematic analysis of clinical experience, quality improvement data, and specialized knowledge and skills of experts from organizations such as, but not limited to, the Academy, Australian Institute of Sport (www.ais.gov.au), US Department of Defense (DoD; http://www.defense.gov/), IOC, National Academy of Science, Engineering and Medicine (National Academies; www.nationalacademies.org), USOPC as basis of practice	X	X	X
	2.7B	Integrates research findings and evidence into peer-reviewed publications and recommendations for practice		X	X
	2.7C	Mentors others in developing skills in accessing and critically analyzing research for application to practice			X
	2.7D	Integrates evidence and best practices into program and organization policies, procedures, and practices			X
2.8	**Participates in peer review of others as applicable to role and responsibilities**		**X**	**X**	**X**
	2.8A	Participates in peer evaluation, including but not limited to peer supervision, clinical chart review, professional practice, and performance evaluations, as applicable	X	X	X
	2.8B	Serves as an editorial board member or reviewer for professional organizations, journals, or books		X	X
	2.8C	Serves as an editor of a journal or other scholarly work, including but not limited to professional articles, chapters, and books			X
2.9	**Mentors and/or precepts others**		**X**	**X**	**X**
	2.9A	Seeks out mentors in SHP nutrition	X	X	X
	2.9B	Serves as mentor and/or preceptor for nutrition and dietetics students or interns; seeks guidance as needed	X	X	X
	2.9C	Develops mentoring or internship opportunities for nutrition and dietetics students or interns and practitioners, and other sports and health care professionals as appropriate		X	X
	2.9D	Directs and implements internships and mentoring programs for nutrition and dietetics students or interns and practitioners, and other sports and health care professionals as appropriate			X
2.10	**Pursues opportunities (education, training, credentials, certifications) to advance practice in accordance with laws and regulations, and requirements of practice setting**		**X**	**X**	**X**

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 2: Competence and Accountability					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
	2.10A	Completes or is currently enrolled in undergraduate or graduate-level courses in sport nutrition, exercise physiology, or kinesiology	X	X	X
	2.10B	Completes SHP nutrition-applicable continuing education courses; attends webinars and SHP-applicable conferences	X	X	X
	2.10C	Pursues documented hours applicable toward SHP nutrition-related certifications or credentials, such as CSSD and IOC Diploma in Sports Nutrition	X	X	X
	2.10D	Obtains and maintains a general fitness credential (eg, Certified Personal Trainer, Group Fitness Instructor)	X	X	X
	2.10E	Participates in SHP-specific professional organizations	X	X	X
	2.10F	Obtains target population-specific physical training credential (eg, Triathlon or Weightlifting Level 1 Coach, Tactical Strength and Conditioning Facilitator)		X	X
	2.10G	Obtains advanced SHP-relevant certifications (eg, Certified Strength and Conditioning Specialist, International Society for the Advancement of Kinanthropometry)			X
	2.10H	Assumes high-level leadership positions in occupational or SHP-specific professional organizations (eg, national elected positions, board member, or chair)			X

Examples of Outcomes for Standard 2: Competence and Accountability

- Practice reflects:
  - Code(s) of ethics (eg, Academy or CDR, other national organizations, or employer code of ethics)
  - Scope of Practice, Standards of Practice, and Standards of Professional Performance
  - Evidence-based practice and best practices
  - Commission on Dietetic Registration Essential Practice Competencies and Performance Indicators
- Practice incorporates successful strategies for interactions with individuals or groups from diverse cultures and backgrounds
- Competence is demonstrated and documented
- Services provided are safe, effective, and athlete-/professional-centered
- Self-evaluations are conducted regularly to reflect commitment to lifelong learning and professional development and engagement
- Professional development needs are identified and pursued
- Directed learning is demonstrated
- Relevant opportunities (education, training, credentials, certifications) are pursued to advance practice
- Commission on Dietetic Registration recertification requirements are met
- Leadership roles are pursued in professional organization(s)

Standard 3: Provision of Services
The registered dietitian nutritionist (RDN) provides safe, quality service based on customer expectations and needs, and the mission, vision, principles, and values of the organization or business.
Rationale:
Quality programs and services are designed, executed, and promoted based on the RDN's knowledge, skills, experience, judgment, and competence in addressing the needs and expectations of the organization or business and its customers.

Indicators for Standard 3: Provision of Services						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
3.1	**Contributes to or leads in development and maintenance of programs or services that address needs of the customer or target population(s)**			**X**	**X**	**X**
	3.1A	**Aligns program or service development with the mission, vision, principles, values, and service expectations and outputs of the organization or business**		**X**	**X**	**X**
		3.1A1	Participates in SHP nutrition program or practice in compliance with evidence-based guidelines and best business management practices; seeks assistance as needed	X	X	X
		3.1A2	Develops or maintains SHP nutrition program or practice in compliance with evidence-based guidelines and best business management practices; seeks assistance as needed		X	X
		3.1A3	Develops programs aligned with organization goals, mission, and vision			X
	3.1B	**Uses the needs, expectations, and desired outcomes of the customers or populations (eg, athletes/professionals, families, community, decision makers, administrators, client organization[s]) in program or service development**		**X**	**X**	**X**
		3.1B1	Participates in SHP nutrition program and service planning (eg, business planning and organization program development)	X	X	X
		3.1B2	Integrates anticipated needs, identified goals, and objectives into program development and delivery; engages in long-term strategic planning		X	X
		3.1B3	Leads in strategic and operational planning, implementation, and monitoring of SHP nutrition programs and services			X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 3: Provision of Services						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
	3.1C	**Makes decisions and recommendations that reflect stewardship of time, talent, finances, and environment**		**X**	**X**	**X**
		3.1C1	Shapes, modifies, and adapts program and service delivery in alignment with budget requirements and priorities		X	X
		3.1C2	Emphasizes the application of SHP nutrition components (Figure 6) to organization and population-level programs			X
	3.1D	**Proposes programs and services that are customer-centered, culturally appropriate, and minimize disparities**		**X**	**X**	**X**
		3.1D1	Uses evidence-based guidelines, best practices, and national and international guidelines (eg, Academy, Dietitians of Canada, ACSM, National Athletic Trainers Associations [NATA; www.nata.org], National Academies, IOC) in the delivery of nutrition services and physical activity recommendations	X	X	X
		3.1D2	Develops nutrition programs, protocols, and policies based on evidence-based guidelines, best practices, and national and international guidelines (eg, Academy, Dietitians of Canada, ACSM, NATA, National Academies, IOC)		X	X
		3.1D3	Directs the development of nutrition programs, protocols, and policies based on evidence-based guidelines, best practices, and national and international guidelines (eg, Academy, Dietitians of Canada, ACSM, NATA, National Academies, IOC)			X
3.2	**Promotes public access and referral to credentialed nutrition and dietetics practitioners for quality food and nutrition programs and services**			**X**	**X**	**X**
	3.2A	**Contributes to or designs referral systems that promote access to qualified, credentialed nutrition and dietetics practitioners**		**X**	**X**	**X**
		3.2A1	Ensures that RDNs are part of an interprofessional approach across collaborative SHP nutrition programs and efforts	X	X	X
		3.2A2	Creates referral tools and processes		X	X
		3.2A3	Directs and manages referral process and system, including establishing agreements and developing or modifying referral systems with SHP partners			X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 3: Provision of Services						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
	3.2B	**Refers customers to appropriate providers when requested services or identified needs exceed the RDN's individual scope of practice**		**X**	**X**	**X**
		3.2B1	Receives referrals from and refers customers to sports medicine and other SHP staff (eg, physician, sport psychologist, physical therapist, exercise physiologist, athletic trainer, strength coach, other coaches) as appropriate	X	X	X
		3.2B2	Verifies potential referral provider's or professional's care or services reflects evidence-based information or research and professional standards of practice	X	X	X
		3.2B3	Establishes and maintains networks to support overall care and services for athletes/professionals	X	X	X
		3.2B4	Supports referral sources with curriculum and training regarding SHP nutrition needs of athletes/ professionals		X	X
		3.2B5	Establishes organization processes and policies for internal and external referrals			X
	3.2C	**Monitors effectiveness of referral systems and modifies as needed to achieve desirable outcomes**		**X**	**X**	**X**
		3.2C1	Provides leadership in documenting, evaluating, and updating referral processes		X	X
		3.2C2	Directs and manages referral processes and systems			X
	3.2D	Evaluates the effectiveness of SHP nutrition referral tools (eg, USOPC Sport Dietitian Registry, and state, local affiliate, and other referral mechanisms)			X	X
3.3	**Contributes to or designs customer-centered services**			**X**	**X**	**X**
	3.3A	**Assesses needs, beliefs or values, goals, resources of the customer, and social determinants of health**		**X**	**X**	**X**
		3.3A1	Applies goal setting and behavior change strategies and techniques (eg, stages of change, motivational interviewing techniques) in practice	X	X	X
	3.3B	**Uses knowledge of the customer's or target population's health conditions, cultural beliefs, and business objectives or services to guide design and delivery of customer-centered services**		**X**	**X**	**X**

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/ professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 3: Provision of Services						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
		3.3B1	Applies principles of SHP nutrition to task-specific physical performance and behavior change appropriate for diverse populations	X	X	X
		3.3B2	Recognizes how the athletic or occupation environment, culture, health literacy, and socioeconomic status may influence task-specific physical performance and athlete/professional population use of SHP and health care services	X	X	X
		3.3B3	Adapts practice to meet the needs of ethnically and culturally diverse populations (eg, uses interpreters, selects appropriate levels of interventions, adapts SHP nutrition education or counseling approaches and materials)	X	X	X
		3.3B4	Applies working knowledge of SHP nutrition, evidence-based guidelines, best practices, and clinical judgment and experience to provide customer-centered services when combining multiple intervention approaches		X	X
		3.3B5	Applies advanced and comprehensive knowledge of SHP nutrition, target sport or organization, evidence-based guidelines, best practices, and clinical judgment and experience to determine the most appropriate action plan in complicated, unpredictable, and dynamic situations			X
	3.3C	**Communicates principles of disease prevention and behavioral change appropriate to the athlete/professional population**		**X**	**X**	**X**
		3.3C1	Recognizes athlete/professional population's concepts of illness, injury, and rehabilitation and their cultural beliefs	X	X	X
		3.3C2	Considers organization culture when communicating disease and injury prevention and behavior change strategies		X	X
	3.3D	**Collaborates with athletes/professionals and organizations to set priorities, establish goals, and create athlete/professional-centered action plans to achieve desirable outcomes**		**X**	**X**	**X**
		3.3D1	Consults with coaches and organization leadership to ensure athlete/professional goals, action plans, and desirable outcomes are aligned with organization goals and desirable outcomes, as appropriate		X	X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 3: Provision of Services						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
	3.3E	**Involves customers in decision making**		**X**	**X**	**X**
		3.3E1	Works in partnership with the athletes/professionals to determine goals, changes necessary to achieve goals and metrics of evaluating progress	X	X	X
		3.3E2	Consults with coaches and organization leadership when determining goals, changes to action plans, and metrics of evaluating progress		X	X
3.4	**Executes programs or services in an organized, collaborative, cost-effective, and athlete-/professional-centered manner**			**X**	**X**	**X**
	3.4A	**Collaborates and coordinates with peers, colleagues, stakeholders, and within interprofessional teams**		**X**	**X**	**X**
		3.4A1	Works in partnership with SHP professionals, other health care providers, and ancillary referral sources	X	X	X
		3.4A2	Serves as a consultant for medical nutrition therapy for the SHP population in the management of nutrition-related illnesses and conditions		X	X
	3.4B	**Uses and participates in, or leads in the selection, design, execution, and evaluation of athlete/professional programs and services (eg, nutrition screening system, medical and retail foodservice, electronic health records, interprofessional programs, community education, grant management)**		**X**	**X**	**X**
		3.4B1	Develops and delivers nutrition education programs and services that integrate components of SHP nutrition across all levels of athletes/professionals		X	X
		3.4B2	Develops and manages SHP nutrition programs and education materials based on athlete/professional needs, cultural beliefs, and evidence-based guidelines (eg, Academy, Dietitians of Canada, ACSM, IOC, National Academies, NATA), and available resources		X	X
		3.4B3	Plans, develops, and implements programs for delivery of SHP nutrition services, using evidence-based guidelines and best practices		X	X
		3.4B4	Directs programs delivering SHP nutrition services			X
		3.4B5	Evaluates the appropriateness and validity of emerging SHP nutrition tools			X

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Indicators for Standard 3: Provision of Services						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
	3.4C	**Uses and develops or contributes to selection, design, and maintenance of policies, procedures (eg, discharge planning or transitions of care, disaster and emergency planning), protocols, standards of care, technology resources (eg, HIPAA-compliant telehealth platforms), and training materials that reflect evidence-based practice in accordance with applicable laws and regulations**		**X**	**X**	**X**
		3.4C1	Participates in developing and updating policies and procedures and evidence-based SHP nutrition practice tools	X	X	X
		3.4C2	Develops policies and procedures for quality improvement (eg, identifies or adapts evidence-based practice guidelines, protocols, or tools, skill training and reinforcement, and organizational incentives and supports) tailored to the needs of the organization and athlete/professional populations		X	X
		3.4C3	Leads the process of developing, monitoring, evaluating, and improving protocols, guidelines, and practice tools (eg, Academy, Dietitians of Canada); implements changes as appropriate			X
		3.4C4	Evaluates organization policies and procedures for vulnerabilities in the context of regional and national-level disasters and emergencies (eg, weather events, public health pan-/epidemics, national or homeland security incidences, cyber-attacks)			X
	3.4D	**Uses and participates in or develops processes for order writing and other nutrition-related privileges, in collaboration with the medical staff,[c] or medical director (eg, public health, community, free-standing clinic settings), consistent with state practice acts, federal and state regulations, organization policies, and medical staff rules, regulations, and bylaws**		**X**	**X**	**X**
		3.4D1	**Uses and participates in or leads development of processes for privileges or other facility-specific processes related to (but not limited to) implementing physician or non-physician practitioner[d]-driven delegated orders or protocols, initiating or modifying orders for therapeutic diets, medical foods, dietary supplements (vitamins/minerals), oral nutrition supplements, laboratory tests, medications, and adjustments to fluid therapies or electrolyte replacements; seeks assistance if needed**	**X**	**X**	**X**

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 3: Provision of Services						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
		3.4D2	**Uses and participates in or leads development of processes for privileging for provision of nutrition-related services, including (but not limited to) initiating or modifying orders for medical foods, dietary supplements (vitamins/minerals), and oral nutrition supplements and indirect calorimetry measurements; seeks assistance if needed**	**X**	**X**	**X**
		3.4D3	Uses and participates in or leads development of processes for coordination of care for athletes/professionals with inpatient facilities and/or clinical service providers	X	X	X
	3.4E	**Complies with established billing regulations, organization policies, grant funder guidelines, if applicable to role and setting, and adheres to ethical and transparent financial management and billing practices**		**X**	**X**	**X**
	3.4F	**Communicates with the interprofessional team and referring party consistent with the HIPAA rules for use and disclosure of customer's personal health information**		**X**	**X**	**X**
3.5	**Uses professional, technical, and support personnel appropriately in the delivery of customer-centered care or services in accordance with laws, regulations, and organization policies and procedures**			**X**	**X**	**X**
	3.5A	**Assigns activities, including direct care to athlete/professional, consistent with the qualifications, experience, and competence of professional, technical, and support personnel**		**X**	**X**	**X**
		3.5A1	Ensures consistent nutrition messaging and maintenance of scopes of practice when using health educators, strength and conditioning coaches, physical therapists, sport psychologists, and others as nutrition education extenders		X	X
	3.5B	**Supervises professional, technical, and support personnel**		**X**	**X**	**X**
		3.5B1	Trains professional, technical, and support personnel and evaluates and documents their skills or competence, following organization or program guidelines		X	X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 3: Provision of Services						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
3.6	**Designs and implements food delivery systems to meet the needs of customers**			**X**	**X**	**X**
	3.6A	**Collaborates in or leads the design of food delivery systems to address health care needs and outcomes (including nutrition status), ecological sustainability, and to meet the culture and related needs and preferences of target populations (ie, athletes/professionals, employee groups, visitors to retail venues, schools, community feeding sites, farm-to-institution initiatives, local food banks)**		**X**	**X**	**X**
		3.6A1	Participates in food service planning and delivery in various situations (eg, community sporting events, training tables, eating out or eating on the road while traveling for competition, military dining programs, humanitarian operations, wildfire emergency response plan)		X	X
	3.6B	**Participates in, consults or collaborates with, or leads the development of menus to address health, nutritional, and cultural needs of target population(s) consistent with federal, state, or funding source regulations or guidelines**		**X**	**X**	**X**
		3.6B1	Consults and provides guidance to organizations interested in SHP nutrition approach regarding foods to incorporate into menus, snack options, and beverages for the population(s) served		X	X
	3.6C	**Participates in, consults or collaborates with, or leads interprofessional process for determining medical foods/ nutritional supplements, dietary supplements, enteral and parenteral nutrition formularies, and delivery systems for target population(s)**		**X**	**X**	**X**
		3.6C1	Participates as a member of an interprofessional team to provide guidance to local and regional active and athletic communities regarding dietary supplements and food products and pertinent regulatory issues	X	X	X
		3.6C2	Consults within an interprofessional team to provide guidance regarding dietary supplements and food products that are in compliance with, and those that do not comply with, anti-doping rules, regulations, and procedures of sports organizations and governing bodies (eg, NCAA, NFHS, IOC, USADA, WADA, professional sports) and organization policies (eg, DoD service-specific drug abuse policies)		X	X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/ professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 3: Provision of Services						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
		3.6C3	Promotes and recommends use of dietary supplements based on overarching organization philosophy, vision, and image and sponsor or endorsement exclusivity agreements			X
		3.6C4	Assumes a leadership role within an interprofessional team to interact with national and international sports governing bodies (eg, NCAA, NFHS, IOC, USADA, WADA, professional sports, occupation organizations) regarding anti-doping rules, regulations, and procedures (eg, facilitates communication between USADA and elite athletes and DoD or military services and service members; contributes to anti-doping policy review and evaluation)			X
3.7	**Maintains records of services provided**			**X**	**X**	**X**
	3.7A	**Documents according to organization policies, procedures, standards, and systems, including electronic health records**		**X**	**X**	**X**
		3.7A1	Collects data and documents outcomes relative to compliance with evidence-based guidelines and best practices	X	X	X
		3.7A2	Maintains documentation as mandated by applicable regulatory agencies, accrediting or credentialing bodies, local, state, and federal regulations and/or laws, organization policies; and consistent with the Nutrition Care Process where appropriate	X	X	X
		3.7A3	Uses electronic health records within the worksite as appropriate	X	X	X
	3.7B	**Implements data management systems to support interoperable data collection, maintenance, and utilization**		**X**	**X**	**X**
	3.7C	**Uses data to document outcomes of services (ie, staff productivity, cost/benefit, budget compliance, outcomes, quality of services) and provide justification for maintenance or expansion of services**		**X**	**X**	**X**
		3.7C1	Shares program outcomes and impact with organization, athletes/professionals, or community participants	X	X	X
		3.7C2	Provides structure and systems for staff to create reports to identify program outcomes and gaps		X	X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 3: Provision of Services						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
		3.7C3	Analyzes and uses data to communicate value of conventional nutrition and SHP nutrition services in relation to athlete/professional population and organization outcomes and goals		X	X
		3.7C4	Collaborates with interprofessional team members to articulate contribution of SHP nutrition services to overall team outcomes			X
	3.7D	**Uses data to demonstrate program or service achievements and compliance with accreditation standards, laws, and regulations**		**X**	**X**	**X**
3.8	**Advocates for provision of quality food and nutrition services as part of public policy**			**X**	**X**	**X**
	3.8A	**Communicates with policy makers regarding the benefit or cost of quality food and nutrition services**		**X**	**X**	**X**
		3.8A1	Participates in legislative and policy-making activities that influence SHP nutrition services and practices	X	X	X
		3.8A2	Participates in national-level governing body (eg, USOPC, DoD) workgroups and task forces that influence SHP nutrition services and practices		X	X
		3.8A3	Provides leadership in advocacy activities or issues; authors scholarly work and delivers presentations on topic; networks with other advocacy-oriented parties and organizations			X
	3.8B	**Advocates in support of food and nutrition programs and services for populations with special needs and chronic conditions (eg, paralympic population)**		**X**	**X**	**X**
	3.8C	**Advocates for protection of the public through multiple avenues of engagement (eg, legislative action, establishing effective relationships with elected leaders and regulatory officials, participation in various Academy committees, workgroups and task forces, Dietetic Practice Groups, Member Interest Groups, and State Affiliates)**		**X**	**X**	**X**

Examples of Outcomes for Standard 3: Provision of Services

- Program or service design and systems reflect organization or business mission, vision, principles, values, and customer needs and expectations
- Customers participate in establishing program or service goals and customer-focused action plans or nutrition interventions (eg, in-person or via telehealth)

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

- Customer-centered needs and preferences are met
- Customers are satisfied with services and products
- Customers have access to food assistance
- Customers have access to food and nutrition services
- Foodservice system incorporates sustainability practices addressing energy and water use and waste management
- Menus reflect the cultural, health, and/or nutritional needs of target population(s) and consideration of ecological sustainability
- Evaluations reflect expected outcomes and established goals
- Effective screening and referral services are established or implemented as designed
- Professional, technical, and support personnel are supervised when providing nutrition care to customers
- Advocates for the provision of SHP nutrition services
- Ethical and transparent financial management and billing practices are used per role and setting

Standard 4: Application of Research
The registered dietitian nutritionist (RDN) applies, participates in, and/or generates research to enhance practice. Evidence-based practice incorporates the best available research or evidence and information in the delivery of nutrition and dietetics services.
Rationale:
Application, participation, and generation of research promote improved safety and quality of nutrition and dietetics practice and services.

Indicators for Standard 4: Application of Research						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
4.1	**Reviews best available research or evidence and information for application to practice**			**X**	**X**	**X**
	4.1A	**Understands basic research design and methodology**		**X**	**X**	**X**
		4.1A1	Demonstrates understanding of research design and methodology, data collection, interpretation of results, application, and use of the Academy of Nutrition and Dietetics Evidence Analysis Library (www.eatright.org) (EAL; www.andeal.org)	X	X	X
	4.1B	Identifies and reads evidence-based information from multiple reputable disciplines and sources (eg, guidelines, practice guidelines, and related resources)		X	X	X
	4.1C	Uses the EAL as a resource in writing or reviewing research papers		X	X	X
	4.1D	Demonstrates understanding of current research, trends, and epidemiological surveys in SHP nutrition, SHP nutrition education, and related areas of exercise science			X	X
	4.1E	Interprets current research in SHP nutrition and related areas and applies to professional practice as appropriate			X	X

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Indicators for Standard 4: Application of Research					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
	4.1F	Identifies key health and performance questions and uses systematic methods to apply evidence-based guidelines to answer questions		X	X
	4.1G	Demonstrates the professional judgment and critical thinking skills required to evaluate the strength of original research and evidence-based guidelines relevant to SHP nutrition practice			X
	4.1H	Functions as a primary or senior author or reviewer of research and organization position papers and other scholarly work			X
4.2	**Uses best available research or evidence and information as the foundation for evidence-based practice**		**X**	**X**	**X**
	4.2A	Follows evidence-based practice guidelines (eg, EAL, Academy, Dietitians of Canada, ACSM, National Academies, IOC) to provide quality care for athletes/professionals	X	X	X
	4.2B	Systematically reviews and applies the best available research where evidence-based practice guidelines for SHP nutrition are not established		X	X
	4.2C	Follows evidence-based practice guidelines to provide safe, effective SHP nutrition services specific to activity or position, health status, age, gender, skill or fitness level of the target population, and environment		X	X
	4.2D	Follows evidence-based practice guidelines to provide safe, effective SHP nutrition services for athletes/professionals who are balancing multiple situations, complications, or medical conditions			X
4.3	**Integrates best available research or evidence and information with best practices, clinical and managerial expertise, and customer values**		**X**	**X**	**X**
	4.3A	Identifies and uses evidence-based policies, procedures, and resources for SHP nutrition practice	X	X	X
	4.3B	Develops and implements evidence-based policies, procedures, and resources for SHP nutrition practice		X	X
	4.3C	Directs the integration of evidence-based policies, procedures, and resources into organization SHP nutrition practice			X
4.4	**Contributes to the development of new knowledge and research in nutrition and dietetics**		**X**	**X**	**X**
	4.4A	Participates in efforts to extend research to practice through journal clubs, professional supervision, and the Academy's Research workgroups (eg, EAL)	X	X	X
	4.4B	Participates in interprofessional research teams identifying research issues or questions and collaborative research activities related to SHP nutrition	X	X	X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 4: Application of Research					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
	4.4C	Uses evidence-based guidelines, best practices, and clinical or practice experience to generate new knowledge and develop guidelines, programs, and policies in SHP nutrition		X	X
	4.4D	Participates in practice-based research networks (eg, Nutrition Research Network) and the development and/or implementation of practice-based research and peer-reviewed publications		X	X
	4.4E	Functions as a primary or senior author of research or organization position papers (eg, Academy, ACSM, National Strength and Conditioning Association [http://www.nsca-lift.org]), and other scholarly work			X
	4.4F	Serves as an advisor, mentor, preceptor, and/or committee member for graduate-level research			X
	4.4G	Serves as SHP nutrition representative on organization research advisory committees			X
4.5	**Promotes application of research in practice through alliances or collaboration with food and nutrition and other professionals and organizations**		**X**	**X**	**X**
	4.5A	Participates as a member or consultant to or investigator in collaborative research teams that examine relationships among nutrition, task-specific physical performance, and career longevity		X	X
	4.5B	Builds relationships among researchers and decision-makers to influence policy development and to translate evidence-based guidelines into SHP nutrition practice		X	X
	4.5C	Serves as a primary or senior investigator in collaborative research teams that examines relationships among nutrition, task-specific physical performance, and career longevity			X

Examples of Outcomes for Standard 4: Application of Research

- Evidence-based practice, best practices, clinical and managerial expertise, and customer values are integrated in the delivery of nutrition and dietetics services
- Customers receive appropriate services based on the effective application of best available research or evidence and information
- Peer-reviewed publications related to SHP nutrition demonstrate contribution of SHP RDNs
- Best available research or evidence and information is used as the foundation of evidence-based practice

Standard 5: Communication and Application of Knowledge
The registered dietitian nutritionist (RDN) effectively applies knowledge and expertise in communications.
Rationale:
The RDN works with others to achieve common goals by effectively sharing and applying unique knowledge, skills, and expertise in food, nutrition, dietetics, and management services.

Indicators for Standard 5: Communication and Application of Knowledge						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
5.1	**Communicates and applies current knowledge and information based on evidence**			**X**	**X**	**X**
	5.1A	**Demonstrates critical thinking and problem-solving skills when communicating with others**		**X**	**X**	**X**
	5.1B	Translates evidence-based research and other sources of evidence to practical application in communications		X	X	X
	5.1C	Demonstrates critical thinking, reflection, and problem-solving skills (eg, uses evidence-based guidelines and selects best format for presentation) in all forms of communications		X	X	X
	5.1D	Uses clinical judgment and experience to integrate current research- and evidence-based guidelines in communications with special populations (eg, disabled athletes and military service members) and in addressing medical nutrition therapy issues			X	X
	5.1E	Uses clinical judgment and experience to integrate current research and evidence-based guidelines into organization policies, procedures, and practices				X
5.2	**Selects appropriate information and the most effective communication method or format that considers customer-centered care and the needs of the individual, group, or population**			**X**	**X**	**X**
	5.2A	**Uses communication methods (ie, oral, print, one-on-one, group, visual, electronic, and social media) targeted to various audiences**		**X**	**X**	**X**
	5.2B	**Uses information technology to communicate, disseminate, manage knowledge, and support decision-making**		**X**	**X**	**X**
		5.2B1	Uses the Nutrition Care Process, EAL, and the Academy's electronic Sports Nutrition Care Manual when making decisions about athlete/professional care	X	X	X
		5.2B2	Contributes to the advancement of technology/informatics (eg, informatics research, software program design) in SHP nutrition		X	X

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Indicators for Standard 5: Communication and Application of Knowledge						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
		5.2B3	Leads in the advancement of technology or informatics (eg, informatics research, software program design) in SHP nutrition			X
5.3	**Integrates knowledge of food and nutrition with knowledge of health, culture, social sciences, communication, informatics, sustainability, and management**			**X**	**X**	**X**
	5.3A	Integrates new scientific knowledge and SHP nutrition experience into practice in new and varied contexts			X	X
	5.3B	Leads the integration of new scientific knowledge and professional judgment in SHP nutrition into practice for the most advanced nutrition problems or in new research methodologies				X
5.4	**Shares current, evidence-based knowledge, and information with various audiences**			**X**	**X**	**X**
	5.4A	**Guides customers, families, students, and interns in the application of knowledge and skills**		**X**	**X**	**X**
		5.4A1	Develops action plans for athletes/professionals that include specific actions or changes and timelines	X	X	X
		5.4A2	Serves on planning committees or task forces to develop continuing education, activities, and programs in SHP nutrition for students and practitioners		X	X
		5.4A3	Participates in updating the Academy's EAL and Sports Nutrition Care Manual in SHP nutrition and related areas		X	X
		5.4A4	Develops and updates SHP nutrition education materials (eg, e-books, e-manuals, webinars, web-based tools, social media, informatics)		X	X
		5.4A5	Contributes to the education and professional development of RDNs, students, and SHP professionals through formal and informal teaching and mentoring		X	X
		5.4A6	Develops mentor and preceptor programs and interprofessional learning opportunities in SHP nutrition		X	X
	5.4B	**Assists individuals and groups to identify and secure appropriate and available educational and other resources and services**		**X**	**X**	**X**
		5.4B1	Recommends current, credible SHP nutrition education resources (eg, USOPC sports nutrition resources and fact sheets, USADA anti-doping education and resources, DoD Human Performance Resource Center) to individuals and groups	X	X	X

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Indicators for Standard 5: Communication and Application of Knowledge						
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators				**The "X" signifies the indicators for the level of practice**		
Each RDN:				**Competent**	**Proficient**	**Expert**
		5.4B2	Serves as a consultant to organizations (eg, business, industry, government) to address the needs of consumers, SHP professionals, and health providers for SHP nutrition education		X	X
	5.4C	**Uses professional writing and verbal skills in all types of communications**		**X**	**X**	**X**
		5.4C1	Uses professional writing and verbal skills to provide credible information and quotes for the general public (eg, local media, social media, interviews) in various situations (eg, sport and occupational events)	X	X	X
		5.4C2	Authors texts, scholarly work, and authoritative articles for consumers and for SHP professionals		X	X
		5.4C3	Serves as invited reviewer, author, and/or presenter at local, regional, national, or international meetings and to the media		X	X
		5.4C4	Serves as local, regional, national, and international SHP nutrition media spokesperson		X	X
		5.4C5	Serves as an opinion leader in SHP nutrition			X
		5.4C6	Directs and manages professional meetings, workshops, task forces, and conferences			X
	5.4D	**Reflects knowledge of population characteristics in communication methods (eg, literacy and numeracy levels, need for translation of written materials or a translator, communication skills, and learning, hearing or vision disabilities)**		**X**	**X**	**X**
5.5	**Establishes credibility and contributes as a food and nutrition resource within the interprofessional health care and management team, organization, and community**			**X**	**X**	**X**
	5.5A	Communicates with the interprofessional team to promote the use of evidence-based guidelines that integrate food and nutrition with task-specific physical performance		X	X	X
	5.5B	Consults with physicians and other health and sports or performance professionals and interprofessional team members on clinical and other health-related issues		X	X	X
	5.5C	Participates in interprofessional collaborations at a systems level (eg, incorporating SHP nutrition within programs aimed at optimizing training and task-specific physical performance)			X	X

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Indicators for Standard 5: Communication and Application of Knowledge					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
	5.5D	Contributes nutrition-related expertise to national projects and professional organizations (eg, DoD, IOC, IPC, NATA, NCAA, NFHS, National Strength and Conditioning Association [http://www.nsca-lift.org], USADA, WADA) as appropriate		X	X
	5.5E	Promotes the specialized knowledge and skills of the SHP RDN to the interprofessional team		X	X
	5.5F	Consults as an expert or resource on emerging scientific information in SHP nutrition or related field with colleagues or medical and SHP communities			X
	5.5G	Identifies new opportunities for leadership across disciplines to promote SHP nutrition			X
5.6	**Communicates performance improvement and research results through publications and presentations**		**X**	**X**	**X**
	5.6A	Presents evidence-based SHP nutrition research and information to organizations and colleagues	X	X	X
	5.6B	Presents evidence-based SHP nutrition research and information at professional meetings and conferences (eg, local, regional, national, international)		X	X
	5.6C	Authors peer-reviewed articles in SHP nutrition and related areas		X	X
	5.6D	Serves in a leadership role for SHP nutrition-related scholarly work (eg, reviewer, editor, editorial advisory board) and in program planning for conferences (eg, local, regional, national, international)		X	X
	5.6E	Translates research findings for incorporation into development of policies, procedures, and guidelines for SHP nutrition at national and international levels			X
	5.6F	Directs collation of research data into publications (eg, position papers, practice papers, meta-analyses, review articles) and presentations			X
5.7	**Seeks opportunities to participate in and assume leadership roles with local, state, and national professional and community-based organizations (eg, government-appointed advisory boards, community coalitions, schools, foundations, or nonprofit organizations serving the food insecure) providing food and nutrition expertise**		**X**	**X**	**X**
	5.7A	Functions as an SHP nutrition resource as an active member of local, state, or national organizations	X	X	X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 5: Communication and Application of Knowledge					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
	5.7B	Serves on SHP nutrition or related committees, task forces, and panels (local, regional, national, and international) for SHP and health care professionals		X	X
	5.7D	Seeks and generates opportunities (local, regional, state, national, international levels) to integrate SHP nutrition practices and programs within larger systems (eg, DoD, IOC, IPC, NCAA, USADA, WADA, USOPC)		X	X
	5.7D	Manages and directs the integration of SHP nutrition principles within larger systems (eg, DoD, IOC, NCAA, USADA, USOPC)			X

Examples of Outcomes for Standard 5: Communication and Application of Knowledge

- Expertise in food, nutrition, dietetics, and management is demonstrated and shared
- Interoperable information technology is used to support practice
- Effective and efficient communications occur through appropriate and professional use of e-mail, texting, social media tools, applications, and videoconference tools
- Individuals, groups, and stakeholders:
  - Receive current and appropriate information and customer-centered service
  - Demonstrate understanding of information and behavioral strategies received
  - Know how to obtain additional guidance from the RDN or other RDN-recommended resources
- Leadership is demonstrated through active professional and community involvement

Standard 6: Utilization and Management of Resources

The registered dietitian nutritionist (RDN) uses resources effectively and efficiently.

Rationale:

The RDN demonstrates leadership through strategic management of time, finances, facilities, supplies, technology, and natural and human resources.

Indicators for Standard 6: Utilization and Management of Resources					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
6.1	**Uses a systematic approach to manage resources and improve outcomes**		**X**	**X**	**X**
	6.1A	Implements administratively sound programs (eg, Nutrition Care Process protocols, food quality and food safety, SHP nutrition counseling, and education)	X	X	X

(continued on next page)

Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 6: Utilization and Management of Resources					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
	6.1B	Participates in operational planning of food and nutrition programs and services (eg, meals and menu planning, delivery, and execution; food systems consultation; Nutrition Care Process; nutrition education, program planning and development)	X	X	X
	6.1C	Collaborates with medical, sports, administrative, military, and food systems staffs in achieving desired outcomes, securing resources and services		X	X
	6.1D	Directs medical, sports, administrative, military, and food systems staffs in achieving desired outcomes, securing resources and services			X
6.2	**Evaluates management of resources with the use of standardized performance measures and benchmarking as applicable**		**X**	**X**	**X**
	6.2A	**Uses the Standards of Excellence Metric Tool to self-assess quality in leadership, organization, practice, and outcomes for an organization (www.eatrightpro.org/excellencetool)**	**X**	**X**	**X**
	6.2B	Manages effective delivery of nutrition programs and services (eg, business and marketing planning, program administration, education program delivery, materials development) to related SHP nutrition programs		X	X
	6.2C	Directs or manages business and strategic planning for the design and delivery of nutrition services in SHP programs in various settings (eg, clinic, cafeteria, travel, corporate, military, research, artistic performance)			X
6.3	**Evaluates safety, effectiveness, efficiency, productivity, sustainability practices, and value while planning and delivering services and products**		**X**	**X**	**X**
	6.3A	Participates in evaluations, selection, and implementation of services and products (eg, surveys, data collection)	X	X	X
	6.3B	Manages the evaluation of services and products (eg, food quality, food safety, team meals, military dining, sports and dietary supplements, sports nutrition products)		X	X
	6.3C	Evaluates the following at the systems level: safety, effectiveness, and budgeting, in planning and delivering nutrition products and services			X
6.4	**Participates in quality assurance and performance improvement and documents outcomes and best practices relative to resource management**		**X**	**X**	**X**
	6.4A	Applies quality assurance and performance improvement to budget management, inventory tracking, ordering and distribution, negotiations for compensation and additional resources (staff, funding, supplies)	X	X	X

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Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).

Indicators for Standard 6: Utilization and Management of Resources					
Bold font indicators are Academy Core RDN Standards of Professional Performance indicators			**The "X" signifies the indicators for the level of practice**		
Each RDN:			**Competent**	**Proficient**	**Expert**
	6.4B	Proactively and systematically anticipates needs, identifies goals and objectives, and engages in long-term strategic planning		X	X
	6.4C	Anticipates outcomes and consequences of different approaches and makes necessary modifications to achieve desired outcomes		X	X
	6.4D	Partners with relevant SHP professionals to assess return on investment of services and programs in accordance with continuous quality improvement systems			X
6.5	**Measures and tracks trends regarding internal and external customer outcomes (eg, satisfaction, key performance indicators)**		**X**	**X**	**X**
	6.5A	Communicates the need for change based on collected data	X	X	X
	6.5B	Implements, monitors, and evaluates changes based on collected data		X	X

Examples of Outcomes for Standard 6: Utilization and Management of Resources

- Resources are effectively and efficiently managed
- Documentation of resource use is consistent with operational and sustainability goals
- Data are used to promote, improve, and validate services, organization practices, and public policy
- Desired outcomes are achieved, documented, and disseminated
- Key performance indicators are identified and tracked in alignment with organization mission, vision, principles, and values

[a]**Interprofessional**: The term *interprofessional* is used in this evaluation resource as a universal term. It includes a diverse group of team members (eg, physicians, nurses, dietitian nutritionists, pharmacists, psychologists, social workers, strength and conditioning coaches, certified athletic trainers, and occupational and physical therapists), depending on the needs of the athlete/professional. Interprofessional could also mean interdisciplinary or multidisciplinary.

[b]**PROMIS**: The Patient-Reported Outcomes Measurement Information System (*PROMIS*) (https://commonfund.nih.gov/promis/index) is a reliable, precise measure of patient-reported health status for physical, mental, and social well-being. *PROMIS* is a web-based resource and is publicly available.

[c]**Medical staff**: A *medical staff* is composed of doctors of medicine or osteopathy and may in accordance with state law, including scope of practice laws, include other categories of physicians, and nonphysician practitioners who are determined to be eligible for appointment by the governing body.[6]

[d]**Non-physician practitioner**: A *nonphysician practitioner* may include a physician assistant, nurse practitioner, clinical nurse specialist, certified registered nurse anesthetist, certified nurse-midwife, clinical social worker, clinical psychologist, anesthesiologist's assistant, qualified dietitian, or qualified nutrition professional. Disciplines considered for privileging by a facility's governing body and medical staff must be in accordance with state law.[6,7]

Figure 2. *(continued)* Standards of Professional Performance for Registered Dietitian Nutritionist in Sports and Human Performance Nutrition. Note: The term *customer* is used in this evaluation resource as a universal term. Customer could also mean athlete/professional; client or patient; client, patient, or customer; participant; consumer; or any individual, group, or organization for whom the RDN provides service. Sub-indicators refer to "in the context of the sports and human performance nutrition" (ie, majority of responsibilities or actions are not intended to be entry level).